

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Daily Note

Visit No: 24

Date of Daily Note: 02/11/2026

Insurance Name: ATA/Holista

Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025

Date of Injury/Onset: 08/19/2023

EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand

Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports finger feels about the same; continues stiffness limiting motion. Functionally using it for daily tasks with focus on fine tasks (e.g., chapstick). Elbows okay overall.

Patient Presentation

Patient/Caregiver Report: Able to perform work tasks with the hand; continues practicing fine motor use (chapstick). Notes some difficulty with tasks requiring twisting (unscrewing IV tubing).

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Improve index finger motion and fine motor function needed for work and daily activities.

Additional Comments: Reports slight improvement in measured finger motion since last visit (VIP 60 to 62). Elbow asymptomatic except for minor bump on car door; anticipates morning awareness but no ongoing issue described.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Undertaking a single task

Comments: Finger ROM stiffness persists; elbow symptoms not limiting activities.

Additional Comments: Continues to intentionally use the index finger across tasks to improve function.

Pain Presentation

Pain Location(s): Right index finger; elbow only sore after accidental bump.

Current Pain Level: 1

Wong-Baker FACES Pain Rating Scale: (2) Hurts little bit

Pain Description: Minimal pain; primary issue is stiffness limiting fine motor tasks.

Alleviating Factors: Activity modification, splinting at night, passive motion on waking, and PRN anti-inflammatory use.

Exacerbating Factors: Twisting motions such as unscrewing IV tubing.

Interventions/Physical Therapy to Address Pain: Continues scar tissue work and tendon gliding/"walking" exercises focused on finger ROM.

Additional Comments: Notes ongoing practice of fine tasks; tolerates daily activities and work tasks overall.

Current Medications

Refer to eDocs

Current Supplements: PRN anti-inflammatory (patient-reported).

Additional Comments: Aims to minimize medication use but notes better symptom control when taking it.

Occupation(s)

Work Classification

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Job Description Referenced? No

Additional Comments: Works in a hand-intensive role involving tasks such as spiking bags of fluid and handling IV tubing; generally managing but twisting tasks can be difficult.

OBJECTIVE

Objective Findings

Summary: Index finger ROM: PIP flexion to ~52 actively; extension lag ~-6. Fingers overall feel "kind of the same." Elbows asymptomatic with activity; patient modifying exercises and tolerating chest/triceps work. Morning stiffness present in finger; improves after splinting and passive motion.

Additional Comments: Clinician notes small persistent PIP contracture; passive motion near full with relative active limitation, consistent with scar adherence. Night LMV splint continued. Patient performing "finger walking" exercises. Workouts modified to avoid full extension loads; able to hold weights with both hands now. Back symptoms provoked by forward folds/downward dog; improving.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Dominant Hand: Ambidextrous

Pinch Test Instrument: Pinch gauge

Lateral Pinch	Meas. 1	Meas. 2	Units	Coefficient of Variance	Average
Left	16	13	lbs	10.34	14.50
Right	17		lbs	0.00	17.00

Comments: Lateral pinch increased compared to prior; noted improvement bilaterally.

Tip Pinch/ Pincer	Meas. 1	Units	Coefficient of Variance	Average
Left	15	lbs	0.00	15.00
Right	17	lbs	0.00	17.00

Comments: Tip pinch increased on both sides compared to prior.

Tripod Pinch	Meas. 1	Units	Coefficient of Variance	Average
Left	20	lbs	0.00	20.00
Right	18	lbs	0.00	18.00

Comments: Tripod pinch showed slight improvement bilaterally.

Grip Test Instrument: Jamar dynamometer

Power Grip	Meas. 1	Meas. 2	Meas. 3	Units	Coefficient of Variance	Average
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Left	68	67	64	lbs	2.56	66.33
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Right	70	70	70	lbs	0.00	70.00
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Comments: Slight decrease in grip strength noted; right side remained consistent across trials.

Repeated Grip **Units**

Left	lbs
-------------	-----

Right	lbs
--------------	-----

Rapid Exchange **Units**

Left	lbs
-------------	-----

Right	lbs
--------------	-----

Five Level Grip	Meas. 3	Meas. 4	Meas. 5	Units	Coefficient of Variance	Average
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Left				lbs		
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Right	68	70	70	lbs	1.36	69.33
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Comments: Grip strength showed slight decrease compared to prior but remains consistent side-to-side; clinician noted repetitive grip strengthening may not be ideal given inflammatory condition.

Additional Comments: Pinch measurements increased: noted values of 17 and increase from 9 to 13 on pinch tests.

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (3)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). Mild difficulty (2)

3: Carry a shopping bag or briefcase. Mild difficulty (2)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. Moderate difficulty (3)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Mild difficulty (2)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Slightly (2)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem? Slightly limited (2)

9: Arm, shoulder, or hand pain. Mild (2)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? No difficulty (1)

Disability/Symptom Score: 20

Additional Comments: Patient reports nighttime wind disturbance causing poor sleep; modifies exercise to reduce strain; uses finger splint intermittently due to numbness when worn too long.

Outcomes Disability/Symptom Score 20.455/100

Observation

Description: PIP contracture persists; volar incision not discussed today.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D3 (middle finger)

Metacarpophalangeal (MCP) Joint: 5.8 cm

Proximal Phalanx (P1): 5.3 cm

Proximal Interphalangeal (PIP) Joint: 5.7 cm

Intermediate Phalanx (P2): 5.4 cm

Comments: Measured today; visual swelling minimal but persistent puffiness noted.

2. **Location:** Right D2 (index finger)

Metacarpophalangeal (MCP) Joint: 6.2 cm

Proximal Phalanx (P1): 6 cm

Proximal Interphalangeal (PIP) Joint: 6 cm

Intermediate Phalanx (P2): 5.7 cm

Comments: Index finger measurements about the same as prior.

Additional Comments: Edema primarily at right middle finger today; side-to-side comparison shows right more swollen than left at corresponding points.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Extension	D2 Flexion
------------------	-----------------	---------------

Metacarpophalangeal (MCP) Joint		80 °
---------------------------------	--	------

Proximal Interphalangeal (PIP) Joint	-6 °	98 °
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Distal Interphalangeal (DIP) Joint		60 °
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AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
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Metacarpophalangeal (MCP) Joint		90 °
---------------------------------	--	------

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Proximal

Interphalangeal (PIP) -6 ° 80 °

Joint

Distal

Interphalangeal (DIP) 63 °

Joint

Left Hand DPCs	Third Digit	Fourth Digit	Comments
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Finger Tip to Distal Palmar Crease (DPC)	-6 cm	60 cm	Index approximately -6 today; DIP noted as main limitation around 60; hinge joint achieved 76 with place-and-hold. No other ROM values discussed.
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PROM of Left Hand

Left Hand DPCs	Comments
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Finger Tip to Distal Palmar Crease (DPC)	Index PIP extension today at -6 degrees; passive motion near full, with residual PIP and DIP stiffness.
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Additional Comments: PIP extension measured at -6 degrees today; previously -8 degrees.

Special Tests

Description: Index finger PIP extension ~-6°; PIP flexion ~52°. Passive flexion near full. Mild morning achiness in finger and elbows; elbows otherwise asymptomatic with activities. Modifying strengthening (e.g., avoiding full extension on triceps work) and able to hold weights in both hands now.

Palpation

Description: Lateral elbow symptoms consistent with lateral epicondyle irritation (patient localizes pain more lateral/top of elbow).

Additional Comments

Continues night LMB extension splinting with morning stiffness requiring passive flexion to initiate motion. Ongoing tendon-gliding and wall/foam walking exercises; advised to continue activity modifications for lateral elbows and avoid provocative positions; progressing gym program cautiously every other to every few days.

ASSESSMENT

Diagnosis

Summary: Elbows asymptomatic and patient has returned to most normal activities including light weightlifting. Index finger active DIP flexion measured 62° today (improved from 60°). -10 degree flexion contracture of the index finger.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Patient actively uses finger for fine tasks (e.g., chapstick tops, IV tubing) and household tasks (e.g., garage door battery); reports some difficulty with fine screwing tasks at work.

Summary

Summary of Findings: Elbows without current pain; finger motion slightly improved with active DIP flexion to 62°. Functional use ongoing with mild difficulty in fine motor/screwing tasks.

Therapeutic Outcome: Other

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Complicating Factors: Multiple sites/body parts treated; Limited range of motion/flexibility

Precautions: Post-surgical precautions

Clinical Impressions/Conclusions: Active finger limitations persist but are improving; elbows stable. Continued focused use and exercises expected to support further gains in fine motor function.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to address finger motion limitations impacting fine motor tasks and to guide activity progression.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Continue practicing fine motor tasks relevant to work (e.g., chapstick tops, IV connections) and adjust splint/edging as needed for comfort and function.

Problems & Goals

1. **Problem:** Decrease in function

Status: Active

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Four weeks

Progress: Strength measures improved bilaterally; motion stable; functional tolerance improving with modifications.

Status: Progressing

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Three months

Progress: Increasing grip/pinch strength; still some swelling; activity modifications in place.

Status: Progressing

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: 60 days

Progress: Motion unchanged with desire for more flexion; pain minimal; mild puffiness persists.

Status: Progressing

4. **Type:** Short term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: 30 days

Progress: Functional use improving with exercise modification; minimal pain reported.

Status: Progressing

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Four weeks

Progress: Grip/pinch increased; swelling measurements stable to slightly improved; tolerance better.

Status: Progressing

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Three months

Progress: Elbows reported "okay"; patient modifies lifts/raises; continued monitoring advised.

Status: Progressing

Rehabilitation Potential: Good

Additional Comments: Rehab potential: Good. Patient demonstrating improved strength and stable motion, with ongoing mild swelling and need for activity/splint modifications. Goals overall progressing; continued focus on tendon glide, controlled strengthening, and protection of extensor mechanism is indicated.

Patient/Caregiver Education

Education Provided: Discussed monitoring for index MP EDC tendon subluxation; advised activity modification and avoidance of repetitive heavy gripping; continue blocking/tendon gliding and functional use while protecting joints. Reviewed splint use (night/relative motion/spring) with caution for numbness and option to gently bend against spring for relief; increase wear as tolerated without compromising circulation. Reinforce communication with Rheumatology regarding tendon behavior and inflammatory symptoms.

Additional Comments: Emphasized symptom monitoring, swelling checks, and gradual progression of activities; maintain modifications during classes and work; follow-up with Rheumatology in March or sooner if symptoms escalate.

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: Continue per plan of care.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		
JM to left IF PIP joint for extension		
PROM finger		
Intrinsic stretching		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units

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3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT

License #OT.0002252

Electronically signed by Megan Brunell, OTR, CHT on February
11, 2026 at 11:03 am

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Occupational Therapy Daily Note

Visit No: 23
Date of Daily Note: 01/28/2026
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports finger feels about the same; continues stiffness limiting motion. Functionally using it for daily tasks with focus on fine tasks (e.g., chapstick). Elbows okay overall.

Patient Presentation

Patient/Caregiver Report: Able to perform work tasks with the hand; continues practicing fine motor use (chapstick). Notes some difficulty with tasks requiring twisting (unscrewing IV tubing).

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Improve index finger motion and fine motor function needed for work and daily activities.

Additional Comments: Reports slight improvement in measured finger motion since last visit (VIP 60 to 62). Elbow asymptomatic except for minor bump on car door; anticipates morning awareness but no ongoing issue described.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Undertaking a single task

Comments: Finger ROM stiffness persists; elbow symptoms not limiting activities.

Additional Comments: Continues to intentionally use the index finger across tasks to improve function.

Pain Presentation

Pain Location(s): Right index finger; elbow only sore after accidental bump.

Current Pain Level: 1

Wong-Baker FACES Pain Rating Scale: (2) Hurts little bit

Pain Description: Minimal pain; primary issue is stiffness limiting fine motor tasks.

Alleviating Factors: Activity modification, splinting at night, passive motion on waking, and PRN anti-inflammatory use.

Exacerbating Factors: Twisting motions such as unscrewing IV tubing.

Interventions/Physical Therapy to Address Pain: Continues scar tissue work and tendon gliding/"walking" exercises focused on finger ROM.

Additional Comments: Notes ongoing practice of fine tasks; tolerates daily activities and work tasks overall.

Current Medications

Refer to eDocs

Current Supplements: PRN anti-inflammatory (patient-reported).

Additional Comments: Aims to minimize medication use but notes better symptom control when taking it.

Occupation(s)

Work Classification

Job Description Referenced? No

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Additional Comments: Works in a hand-intensive role involving tasks such as spiking bags of fluid and handling IV tubing; generally managing but twisting tasks can be difficult.

OBJECTIVE

Objective Findings

Summary: Index finger ROM: PIP flexion to ~52 actively; extension lag ~-6. Fingers overall feel "kind of the same." Elbows asymptomatic with activity; patient modifying exercises and tolerating chest/triceps work. Morning stiffness present in finger; improves after splinting and passive motion.

Additional Comments: Clinician notes small persistent PIP contracture; passive motion near full with relative active limitation, consistent with scar adherence. Night LMV splint continued. Patient performing "finger walking" exercises. Workouts modified to avoid full extension loads; able to hold weights with both hands now. Back symptoms provoked by forward folds/downward dog; improving.

Standardized Tests

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Tip Pinch/ Pincer	Meas. 1	Units	Coefficient of Variance	Average
Left	15	lbs	0.00	15.00
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Comments: Tip pinch increased on both sides compared to prior.

Tripod Pinch	Meas. 1	Units	Coefficient of Variance	Average
Left	20	lbs	0.00	20.00
Right	18	lbs	0.00	18.00

Comments: Tripod pinch showed slight improvement bilaterally.

Grip Test Instrument: Jamar dynamometer

Power Grip	Meas. 1	Meas. 2	Meas. 3	Units	Coefficient of Variance	Average
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Left	68	67	64	lbs	2.56	66.33
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Right	70	70	70	lbs	0.00	70.00
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Comments: Slight decrease in grip strength noted; right side remained consistent across trials.

Repeated Grip **Units**

Left	lbs
-------------	-----

Right	lbs
--------------	-----

Rapid Exchange **Units**

Left	lbs
-------------	-----

Right	lbs
--------------	-----

Five Level Grip	Meas. 3	Meas. 4	Meas. 5	Units	Coefficient of Variance	Average
------------------------	----------------	----------------	----------------	--------------	--------------------------------	----------------

Left				lbs		
-------------	--	--	--	-----	--	--

Right	68	70	70	lbs	1.36	69.33
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Comments: Grip strength showed slight decrease compared to prior but remains consistent side-to-side; clinician noted repetitive grip strengthening may not be ideal given inflammatory condition.

Additional Comments: Pinch measurements increased: noted values of 17 and increase from 9 to 13 on pinch tests.

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (3)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). Mild difficulty (2)

3: Carry a shopping bag or briefcase. Mild difficulty (2)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. Moderate difficulty (3)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Mild difficulty (2)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Slightly (2)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem? Slightly limited (2)

9: Arm, shoulder, or hand pain. Mild (2)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? No difficulty (1)

Disability/Symptom Score: 20

Additional Comments: Patient reports nighttime wind disturbance causing poor sleep; modifies exercise to reduce strain; uses finger splint intermittently due to numbness when worn too long.

Outcomes Disability/Symptom Score 20.455/100

Observation

Description: PIP contracture persists; volar incision not discussed today.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D3 (middle finger)

Metacarpophalangeal (MCP) Joint: 5.8 cm

Proximal Phalanx (P1): 5.3 cm

Proximal Interphalangeal (PIP) Joint: 5.7 cm

Intermediate Phalanx (P2): 5.4 cm

Comments: Measured today; visual swelling minimal but persistent puffiness noted.

2. **Location:** Right D2 (index finger)

Metacarpophalangeal (MCP) Joint: 6.2 cm

Proximal Phalanx (P1): 6 cm

Proximal Interphalangeal (PIP) Joint: 6 cm

Intermediate Phalanx (P2): 5.7 cm

Comments: Index finger measurements about the same as prior.

Additional Comments: Edema primarily at right middle finger today; side-to-side comparison shows right more swollen than left at corresponding points.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Extension	D2 Flexion
------------------	-----------------	---------------

Metacarpophalangeal (MCP) Joint		80 °
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Proximal Interphalangeal (PIP) Joint	-6 °	98 °
--------------------------------------	------	------

Distal Interphalangeal (DIP) Joint		60 °
------------------------------------	--	------

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
-----------------	-----------------	---------------

Metacarpophalangeal (MCP) Joint		90 °
---------------------------------	--	------

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Proximal

Interphalangeal (PIP) -6 ° 80 °

Joint

Distal

Interphalangeal (DIP) 63 °

Joint

Left Hand DPCs	Third Digit	Fourth Digit	Comments
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Finger Tip to Distal Palmar Crease (DPC)	-6 cm	60 cm	Index approximately -6 today; DIP noted as main limitation around 60; hinge joint achieved 76 with place-and-hold. No other ROM values discussed.
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PROM of Left Hand

Left Hand DPCs	Comments
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Finger Tip to Distal Palmar Crease (DPC)	Index PIP extension today at -6 degrees; passive motion near full, with residual PIP and DIP stiffness.
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Additional Comments: PIP extension measured at -6 degrees today; previously -8 degrees.

Special Tests

Description: Index finger PIP extension ~-6°; PIP flexion ~52°. Passive flexion near full. Mild morning achiness in finger and elbows; elbows otherwise asymptomatic with activities. Modifying strengthening (e.g., avoiding full extension on triceps work) and able to hold weights in both hands now.

Palpation

Description: Lateral elbow symptoms consistent with lateral epicondyle irritation (patient localizes pain more lateral/top of elbow).

Additional Comments

Continues night LMB extension splinting with morning stiffness requiring passive flexion to initiate motion. Ongoing tendon-gliding and wall/foam walking exercises; advised to continue activity modifications for lateral elbows and avoid provocative positions; progressing gym program cautiously every other to every few days.

ASSESSMENT

Diagnosis

Summary: Elbows asymptomatic; minor bump to elbow without ongoing issues. Index finger active DIP flexion measured 62° today (improved from 60°).

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Patient actively uses finger for fine tasks (e.g., chapstick tops, IV tubing) and household tasks (e.g., garage door battery); reports some difficulty with fine screwing tasks at work.

Summary

Summary of Findings: Elbows without current pain; finger motion slightly improved with active DIP flexion to 62°. Functional use ongoing with mild difficulty in fine motor/screwing tasks.

Therapeutic Outcome: Other

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Complicating Factors: Multiple sites/body parts treated; Limited range of motion/flexibility

Precautions: Post-surgical precautions

Clinical Impressions/Conclusions: Active finger limitations persist but are improving; elbows stable. Continued focused use and exercises expected to support further gains in fine motor function.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to address finger motion limitations impacting fine motor tasks and to guide activity progression.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Continue practicing fine motor tasks relevant to work (e.g., chapstick tops, IV connections) and adjust splint/edging as needed for comfort and function.

Problems & Goals

1. **Problem:** Decrease in function

Status: Active

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Four weeks

Progress: Strength measures improved bilaterally; motion stable; functional tolerance improving with modifications.

Status: Progressing

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Three months

Progress: Increasing grip/pinch strength; still some swelling; activity modifications in place.

Status: Progressing

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: 60 days

Progress: Motion unchanged with desire for more flexion; pain minimal; mild puffiness persists.

Status: Progressing

4. **Type:** Short term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: 30 days

Progress: Functional use improving with exercise modification; minimal pain reported.

Status: Progressing

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Four weeks

Progress: Grip/pinch increased; swelling measurements stable to slightly improved; tolerance better.

Status: Progressing

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Three months

Progress: Elbows reported "okay"; patient modifies lifts/raises; continued monitoring advised.

Status: Progressing

Rehabilitation Potential: Good

Additional Comments: Rehab potential: Good. Patient demonstrating improved strength and stable motion, with ongoing mild swelling and need for activity/splint modifications. Goals overall progressing; continued focus on tendon glide, controlled strengthening, and protection of extensor mechanism is indicated.

Patient/Caregiver Education

Education Provided: Discussed monitoring for index MP EDC tendon subluxation; advised activity modification and avoidance of repetitive heavy gripping; continue blocking/tendon gliding and functional use while protecting joints. Reviewed splint use (night/relative motion/spring) with caution for numbness and option to gently bend against spring for relief; increase wear as tolerated without compromising circulation. Reinforce communication with Rheumatology regarding tendon behavior and inflammatory symptoms.

Additional Comments: Emphasized symptom monitoring, swelling checks, and gradual progression of activities; maintain modifications during classes and work; follow-up with Rheumatology in March or sooner if symptoms escalate.

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: Continue per plan of care.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		
JM to left IF PIP joint for extension		
PROM finger		
Intrinsic stretching		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT

License #OT.0002252

Electronically signed by Megan Brunell, OTR, CHT on January
28, 2026 at 4:51 pm

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
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Occupational Therapy Daily Note

Visit No: 22
Date of Daily Note: 01/21/2026
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports finger feels about the same with stiffness limiting motion; functionally able to use it for daily tasks. Elbows generally asymptomatic during activities. Continues exercises with modifications and feels overall progress is steady but not full.

Patient Presentation

Patient/Caregiver Report: Able to lift weights in both hands again with modifications; performing chest/triceps and yoga with reduced intensity. Focus remains on improving index finger motion over elbow issues.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Improve index finger range of motion while maintaining symptom control during workouts and daily activities.

Additional Comments: Wakes with marked finger stiffness that improves with passive motion; sleeps in night splint. Diagnosed with psoriatic arthritis after prior RA diagnosis; uses anti-inflammatory medication PRN. Back pain improved; downward dog previously aggravated it.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Comments: Finger ROM restricted with morning stiffness; elbow symptoms largely resolved with activity modification.

Additional Comments: Using night extension splint and home exercises; emphasizes using the index finger in daily tasks despite compensations.

Pain Presentation

Pain Location(s): Right index finger; intermittent morning elbow ache.

Current Pain Level: 1

Wong-Baker FACES Pain Rating Scale: (2) Hurts little bit

Pain Description: Minimal pain, primarily stiffness of the index finger with limited PIP/DIP motion; elbows largely asymptomatic with rare mild morning ache.

Alleviating Factors: Activity modification, splinting at night, passive motion on waking, and PRN anti-inflammatory use.

Exacerbating Factors: Full extension activities, heavier curls, and certain yoga positions previously aggravated symptoms.

Interventions/Physical Therapy to Address Pain: Continues scar tissue work and tendon gliding/"walking" exercises focused on finger ROM.

Additional Comments: Reports improved ability to hold weights bilaterally compared to a few weeks prior.

Current Medications

Refer to eDocs

Current Supplements: PRN anti-inflammatory (patient-reported).

Additional Comments: Aims to minimize medication use but notes better symptom control when taking it.

Patient Name: Katherine Holmes
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Occupation(s)

Work Classification

Job Description Referenced? No

Additional Comments: Patient is a nurse; active lifestyle with yoga and lifting.

OBJECTIVE

Objective Findings

Summary: Index finger ROM: PIP flexion to ~52 actively; extension lag ~-6. Fingers overall feel "kind of the same." Elbows asymptomatic with activity; patient modifying exercises and tolerating chest/triceps work. Morning stiffness present in finger; improves after splinting and passive motion.

Additional Comments: Clinician notes small persistent PIP contracture; passive motion near full with relative active limitation, consistent with scar adherence. Night LMV splint continued. Patient performing "finger walking" exercises. Workouts modified to avoid full extension loads; able to hold weights with both hands now. Back symptoms provoked by forward folds/downward dog; improving.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Dominant Hand: Ambidextrous

Pinch Test Instrument: Pinch gauge

Lateral Pinch	Meas. 1	Meas. 2	Units	Coefficient of Variance	Average
Left	16	13	lbs	10.34	14.50
Right	17		lbs	0.00	17.00

Comments: Lateral pinch increased compared to prior; noted improvement bilaterally.

Tip Pinch/ Pincer	Meas. 1	Units	Coefficient of Variance	Average
Left	15	lbs	0.00	15.00
Right	17	lbs	0.00	17.00

Comments: Tip pinch increased on both sides compared to prior.

Tripod Pinch	Meas. 1	Units	Coefficient of Variance	Average
Left	20	lbs	0.00	20.00
Right	18	lbs	0.00	18.00

Comments: Tripod pinch showed slight improvement bilaterally.

Grip Test Instrument: Jamar dynamometer

Patient Name: Katherine Holmes
 Date of Birth: 08/16/1982
 EMR ID: 50300063
 Referring Physician/NPP: Christopher Fender MD

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Power Grip	Meas. 1	Meas. 2	Meas. 3	Units	Coefficient of Variance	Average
Left	68	67	64	lbs	2.56	66.33
Right	70	70	70	lbs	0.00	70.00

Comments: Slight decrease in grip strength noted; right side remained consistent across trials.

Repeated Grip	Units
Left	lbs
Right	lbs
Rapid Exchange	Units
Left	lbs
Right	lbs

Five Level Grip	Meas. 3	Meas. 4	Meas. 5	Units	Coefficient of Variance	Average
Left				lbs		
Right	68	70	70	lbs	1.36	69.33

Comments: Grip strength showed slight decrease compared to prior but remains consistent side-to-side; clinician noted repetitive grip strengthening may not be ideal given inflammatory condition.

Additional Comments: Pinch measurements increased: noted values of 17 and increase from 9 to 13 on pinch tests.

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (3)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). Mild difficulty (2)

3: Carry a shopping bag or briefcase. Mild difficulty (2)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. Moderate difficulty (3)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Mild difficulty (2)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Slightly (2)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem?

Slightly limited (2)

9: Arm, shoulder, or hand pain. Mild (2)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? No difficulty (1)

Disability/Symptom Score: 20

Additional Comments: Patient reports nighttime wind disturbance causing poor sleep; modifies exercise to reduce strain; uses finger splint intermittently due to numbness when worn too long.

Outcomes Disability/Symptom Score 20.455/100

Observation

Description: PIP contracture persists; volar incision not discussed today.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D3 (middle finger)

Metacarpophalangeal (MCP) Joint: 5.8 cm

Proximal Phalanx (P1): 5.3 cm

Proximal Interphalangeal (PIP) Joint: 5.7 cm

Intermediate Phalanx (P2): 5.4 cm

Comments: Measured today; visual swelling minimal but persistent puffiness noted.

2. **Location:** Right D2 (index finger)

Metacarpophalangeal (MCP) Joint: 6.2 cm

Proximal Phalanx (P1): 6 cm

Proximal Interphalangeal (PIP) Joint: 6 cm

Intermediate Phalanx (P2): 5.7 cm

Comments: Index finger measurements about the same as prior.

Additional Comments: Edema primarily at right middle finger today; side-to-side comparison shows right more swollen than left at corresponding points.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Extension	D2 Flexion
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Metacarpophalangeal (MCP) Joint		80 °
---------------------------------	--	------

Proximal

Interphalangeal (PIP) Joint	-6 °	98 °
-----------------------------	------	------

Distal

Interphalangeal (DIP) Joint		60 °
-----------------------------	--	------

AROM of Left Hand

Patient Name: Katherine Holmes
 Date of Birth: 08/16/1982
 EMR ID: 50300063
 Referring Physician/NPP: Christopher Fender MD

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 4045 Wadsworth Blvd Ste 210
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Left Digit AROM	D2	D2
	Extension	Flexion
Metacarpophalangeal (MCP) Joint		90 °
Proximal Interphalangeal (PIP) Joint	-6 °	80 °
Distal Interphalangeal (DIP) Joint		63 °

Left Hand DPCs	Third Digit	Fourth Digit	Comments
Finger Tip to Distal Palmar Crease (DPC)	-6 cm	60 cm	Index approximately -6 today; DIP noted as main limitation around 60; hinge joint achieved 76 with place-and-hold. No other ROM values discussed.

PROM of Left Hand

Left Hand DPCs	Comments
Finger Tip to Distal Palmar Crease (DPC)	Index PIP extension today at -6 degrees; passive motion near full, with residual PIP and DIP stiffness.

Additional Comments: PIP extension measured at -6 degrees today; previously -8 degrees.

Special Tests

Description: Index finger PIP extension ~-6°; PIP flexion ~52°. Passive flexion near full. Mild morning achiness in finger and elbows; elbows otherwise asymptomatic with activities. Modifying strengthening (e.g., avoiding full extension on triceps work) and able to hold weights in both hands now.

Palpation

Description: Lateral elbow symptoms consistent with lateral epicondyle irritation (patient localizes pain more lateral/top of elbow).

Additional Comments

Continues night LMB extension splinting with morning stiffness requiring passive flexion to initiate motion. Ongoing tendon-gliding and wall/foam walking exercises; advised to continue activity modifications for lateral elbows and avoid provocative

positions; progressing gym program cautiously every other to every few days.

ASSESSMENT

Diagnosis

Summary: Elbows currently asymptomatic with modified workouts; mild morning ache only. Index finger similar overall; PIP extension about -6° today and DIP flexion about 52° actively, with near full passive motion suggesting scar adherence limiting active excursion. Continues daily functional use; focus on tendon excursion and motion.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Patient modifies exercises (avoids full extension, adjusts curls), uses LMV/night splint, and performs home walking. Reports morning stiffness; uses anti-inflammatory medication as needed.

Summary

Summary of Findings: Elbows largely non-painful with current activity; brief morning ache only. Index finger with PIP extension near -6° and active DIP flexion about 52° , with good passive motion indicating scar-related limitation. Functional use adequate; patient working toward more composite flexion.

Therapeutic Outcome: Other; Increased joint mobility

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Progress in Treatment: Symptoms decreased and functioning improved

Complicating Factors: Multiple sites/body parts treated

Precautions: Post-surgical precautions

Clinical Impressions/Conclusions: Active limitation primarily due to tendon/scar adherence with near full passive motion; PIP contracture improved to $\sim -6^{\circ}$. Elbows stable with activity modifications. Ongoing targeted exercises expected to yield incremental gains in composite flexion.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to address scar adherence, improve tendon excursion and composite flexion, and monitor/guide activity modifications while maintaining elbow tolerance.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Reinforce DIP isolation with thumb support, place-and-hold, brief putty scraping/pinching; continue LMV/night splint; gradual progression based on morning stiffness and elbow tolerance.

Problems & Goals

1. **Problem:** Decrease in function

Status: Active

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Four weeks

Progress: Strength measures improved bilaterally; motion stable; functional tolerance improving with modifications.

Status: Progressing

2. **Type:** Short term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Six weeks

Progress: Increasing grip/pinch strength; still some swelling; activity modifications in place.

Status: Progressing

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Eight weeks

Progress: Motion unchanged with desire for more flexion; pain minimal; mild puffiness persists.

Status: Progressing

4. **Type:** Short term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

Progress: Functional use improving with exercise modification; minimal pain reported.

Status: Progressing

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Four weeks

Progress: Grip/pinch increased; swelling measurements stable to slightly improved; tolerance better.

Status: Progressing

6. **Type:** Short term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Six weeks

Progress: Elbows reported "okay"; patient modifies lifts/raises; continued monitoring advised.

Status: Progressing

Rehabilitation Potential: Good

Additional Comments: Rehab potential: Good. Patient demonstrating improved strength and stable motion, with ongoing mild swelling and need for activity/splint modifications. Goals overall progressing; continued focus on tendon glide, controlled strengthening, and protection of extensor mechanism is indicated.

Patient/Caregiver Education

Education Provided: Discussed monitoring for index MP EDC tendon subluxation; advised activity modification and avoidance of repetitive heavy gripping; continue blocking/tendon gliding and functional use while protecting joints. Reviewed splint use (night/relative motion/spring) with caution for numbness and option to gently bend against spring for relief; increase wear as tolerated without compromising circulation. Reinforce communication with Rheumatology regarding tendon behavior and inflammatory symptoms.

Additional Comments: Emphasized symptom monitoring, swelling checks, and gradual progression of activities; maintain modifications during classes and work; follow-up with Rheumatology in March or sooner if symptoms escalate.

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: Continue per plan of care.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		

JM to left IF PIP joint for extension

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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Tel: (303) 953-3163 Fax: (303) 245-0726



CPT® Code	Modifier	Units
PROM finger		
Intrinsic stretching		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units
		3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on January 21, 2026 at 12:44 pm

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
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Occupational Therapy Daily Note

Visit No: 21
Date of Daily Note: 01/14/2026
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports elbows are generally okay without symptoms during activities; performing push-ups and yoga. Using right index finger more in daily tasks (e.g., pinching/pulling items) and feels function is adequate though motion seems plateaued; hopes for further improvement over time. Notes occasional transient flares with activities that improve after brief rest.

Patient Presentation

Patient/Caregiver Report: Functionally able to do desired activities with modified intensity; took a lighter week after heavier workouts (yoga, sculpt, core, leg deadlifts) and symptoms improved. Interested in focusing therapy on finger motion over elbow.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Improve finger motion while maintaining ability to perform activities without provoking elbow symptoms.

Additional Comments: Considering nutrition-based evaluation ordered by chiropractor (saliva sensitivity test) and contemplating rheumatology medication; currently using Mobic PRN about three times per week.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Right index finger.

Pain Description: Finger feels similar overall; functional without significant pain but limited by stiffness/ROM. Elbows currently asymptomatic during activity. Brief episode of opposite-sided pain when pulling a freezer drawer, resolved to mild discomfort.

Alleviating Factors: Activity modification and brief rest; lighter activity week improved symptoms; PRN Mobic helps.

Exacerbating Factors: Heavier workout weeks and certain daily tasks (e.g., pulling freezer drawer) can provoke transient discomfort.

Interventions/Physical Therapy to Address Pain: Patient open to focusing upcoming visits on finger ROM work.

Additional Comments: Patient will inform rheumatology about tendon subluxation; surgical option exists but deferred unless necessary.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Summary: Index finger ROM: DIP flexion measured to 60 actively; with place-and-hold reached 76. Extension measured at approximately -8. Patient functionally using hand without significant pain; continues push-ups and yoga without notable elbow symptoms.

Additional Comments: Progress measurements taken: index finger ROM and extension remain about the same; clinician would like a bit more flexion. Grip strength showed slight decrease bilaterally compared side-to-side but remains consistent overall; pinch strength increased (noted examples: from 9 to 13 on one measure). Edema measurements: left/right comparisons recorded with mild puffiness despite visually minimal swelling (examples noted: 6.2 previously and currently at same sites; other sites measured at 6.0, 5.8 vs 6.2, 5.3 vs 5.7). Patient reports intermittent extensor digitorum communis tendon subluxation at the index MP with palpable crossover; clinician observed an event during session and discussed monitoring and potential relative motion orthosis with nighttime extension splinting if recurrent. Patient has been modifying workouts; tolerates shoulder/chest work, forward folds aggravate back. Spring splint use noted (kept in car; worn mornings/evenings); caution about numbness with prolonged wear and option to bend against spring briefly for relief.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Dominant Hand: Right

Pinch Test Instrument: Pinch gauge

Lateral Pinch	Meas. 1	Meas. 2	Units	Coefficient of Variance	Average
Left	16	13	lbs	10.34	14.50
Right	17		lbs	0.00	17.00

Comments: Lateral pinch increased compared to prior; noted improvement bilaterally.

Tip Pinch/ Pincer	Meas. 1	Units	Coefficient of Variance	Average
Left	15	lbs	0.00	15.00
Right	17	lbs	0.00	17.00

Comments: Tip pinch increased on both sides compared to prior.

Tripod Pinch	Meas. 1	Units	Coefficient of Variance	Average
Left	20	lbs	0.00	20.00
Right	18	lbs	0.00	18.00

Comments: Tripod pinch showed slight improvement bilaterally.

Grip Test Instrument: Jamar dynamometer

Patient Name: Katherine Holmes
 Date of Birth: 08/16/1982
 EMR ID: 50300063
 Referring Physician/NPP: Christopher Fender MD

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Power Grip	Meas. 1	Meas. 2	Meas. 3	Units	Coefficient of Variance	Average
Left	68	67	64	lbs	2.56	66.33
Right	70	70	70	lbs	0.00	70.00

Comments: Slight decrease in grip strength noted; right side remained consistent across trials.

Repeated Grip	Units
Left	lbs
Right	lbs
Rapid Exchange	Units
Left	lbs
Right	lbs

Five Level Grip	Meas. 3	Meas. 4	Meas. 5	Units	Coefficient of Variance	Average
Left				lbs		
Right	68	70	70	lbs	1.36	69.33

Comments: Grip strength showed slight decrease compared to prior but remains consistent side-to-side; clinician noted repetitive grip strengthening may not be ideal given inflammatory condition.

Additional Comments: Pinch measurements increased: noted values of 17 and increase from 9 to 13 on pinch tests.

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (0) No pain

1: Open a tight or new jar. No difficulty (1)

2: Do heavy household chores (e.g., wash walls, floors). No difficulty (1)

3: Carry a shopping bag or briefcase. No difficulty (1)

4: Wash your back. Mild difficulty (2)

5: Use a knife to cut food. No difficulty (1)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Mild difficulty (2)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Not at all (1)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem?

Not limited at all (1)

9: Arm, shoulder, or hand pain. None (1)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? No difficulty (1)

Disability/Symptom Score: 5

Additional Comments: Patient reports nighttime wind disturbance causing poor sleep; modifies exercise to reduce strain; uses finger splint intermittently due to numbness when worn too long.

Outcomes Disability/Symptom Score 4.546/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D2 (index finger)

Metacarpophalangeal (MCP) Joint: 5.8 cm

Proximal Phalanx (P1): 5.3 cm

Proximal Interphalangeal (PIP) Joint: 5.7 cm

Intermediate Phalanx (P2): 5.4 cm

Comments: Measured today; visual swelling minimal but persistent puffiness noted.

2. **Location:** Right D3 (middle finger)

Metacarpophalangeal (MCP) Joint: 6.2 cm

Proximal Phalanx (P1): 6 cm

Proximal Interphalangeal (PIP) Joint: 6 cm

Intermediate Phalanx (P2): 5.7 cm

Comments: Index finger measurements about the same as prior.

Additional Comments: Edema primarily at right middle finger today; side-to-side comparison shows right more swollen than left at corresponding points.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM **D2 Flexion**

Metacarpophalangeal (MCP) Joint 80 °

Proximal Interphalangeal (PIP) Joint 98 °

Distal Interphalangeal (DIP) Joint 60 °

AROM of Left Hand

Patient Name: Katherine Holmes
 Date of Birth: 08/16/1982
 EMR ID: 50300063
 Referring Physician/NPP: Christopher Fender MD

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Left Digit AROM	D2	D2
	Extension	Flexion
Metacarpophalangeal (MCP) Joint		90 °
Proximal Interphalangeal (PIP) Joint	-6 °	80 °
Distal Interphalangeal (DIP) Joint		63 °

Left Hand DPCs	Third Digit	Fourth Digit	Comments
			Values referenced during session: index approximately -8; distal interphalangeal joint noted as main limitation around 60; hinge joint achieved 76 with place-and-hold. No other ROM values discussed.
Finger Tip to Distal Palmar Crease (DPC)	-8 cm	60 cm	

PROM of Left Hand

Left Hand DPCs	Comments
Finger Tip to Distal Palmar Crease (DPC)	Index DIP noted as primary limitation; measured 60 degrees; improved to 76 degrees with place-and-hold.

Additional Comments: PIP extension measured at -8 degrees.

Special Tests

Description: Middle finger PIP extension ~-8°; DIP flexion to 76° with place-and-hold. PIP flexion about 60°. Passive flexion near full. Elbow symptoms minimal during activities; occasional pain with certain movements (e.g., pulling freezer drawer).

Palpation

Description: Lateral elbow symptoms consistent with lateral epicondyle irritation (patient localizes pain more lateral/top of elbow).

Additional Comments

Night extension splint adjusted for slightly increased extension; advised Coban under splint to improve fit. Emphasis on blocking exercises to improve tendon glide; flexion strap lower priority. Advised to avoid palm-down/wrist pronated exercises and avoid tight grasping of weights; focus on proximal strengthening. Patient reports stiffness in the morning without pain after night splinting.

ASSESSMENT

Diagnosis

Summary: Elbows generally asymptomatic with current activity; patient performing yoga and push-ups without significant elbow pain. Index finger feels similar overall with functional use improving; patient desires more composite flexion. Measured DIP flexion around 60–76° with place-and-hold; PIP extension about –8°. Patient actively using hand in daily tasks and increasing use. Therapy focus today on index finger tendon excursion and motion.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Patient reports intermittent elbow discomfort with specific activities (e.g., lifting from freezer) but overall improving; prefers focusing visits on finger motion. Using Mobic PRN (~3x/week). Discussing rheumatology medications externally; no changes initiated here.

Summary

Summary of Findings: Elbows largely non-painful during exercise classes; occasional transient discomfort with specific tasks. Index finger with PIP extension near –8° and DIP flexion about 60°, improving to 76° with place-and-hold. Functional pinch/grasp adequate for daily activities; patient seeks additional composite flexion.

Therapeutic Outcome: Other

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Clinical Impressions/Conclusions: Index finger motion limited primarily at DIP with improved flexion on place-and-hold, supporting tendon excursion focus; PIP extension near –8°. Elbows stable with current activity level; occasional task-specific discomfort. Continued targeted exercises expected to yield incremental motion gains.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to improve index finger tendon excursion and composite flexion, monitor tolerance to exercise, and support return to full functional use while managing intermittent elbow symptoms.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Emphasize DIP isolation with thumb support, place-and-hold, and brief putty scraping/pinching; monitor response and adjust frequency of visits to focus on finger as elbow allows.

Problems & Goals

1. **Problem:** Decrease in function

Status: Active

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Four weeks

Progress: Strength measures improved bilaterally; motion stable; functional tolerance improving with modifications.

Status: Progressing

2. **Type:** Short term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

Progress: Increasing grip/pinch strength; still some swelling; activity modifications in place.

Status: Progressing

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Four weeks

Progress: Motion unchanged with desire for more flexion; pain minimal; mild puffiness persists.

Status: Progressing

4. **Type:** Short term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

Progress: Functional use improving with exercise modification; minimal pain reported.

Status: Progressing

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Four weeks

Progress: Grip/pinch increased; swelling measurements stable to slightly improved; tolerance better.

Status: Progressing

6. **Type:** Short term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Progress: Elbows reported "okay"; patient modifies lifts/raises; continued monitoring advised.

Status: Progressing

Rehabilitation Potential: Good

Additional Comments: Rehab potential: Good. Patient demonstrating improved strength and stable motion, with ongoing mild swelling and need for activity/splint modifications. Goals overall progressing; continued focus on tendon glide, controlled strengthening, and protection of extensor mechanism is indicated.

Patient/Caregiver Education

Education Provided: Discussed monitoring for index MP EDC tendon subluxation; advised activity modification and avoidance of repetitive heavy gripping; continue blocking/tendon gliding and functional use while protecting joints. Reviewed splint use (night/relative motion/spring) with caution for numbness and option to gently bend against spring for relief; increase wear as tolerated without compromising circulation. Reinforce communication with Rheumatology regarding tendon behavior and inflammatory symptoms.

Additional Comments: Emphasized symptom monitoring, swelling checks, and gradual progression of activities; maintain modifications during classes and work; follow-up with Rheumatology in March or sooner if symptoms escalate.

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: Focus upcoming visits on the index finger to improve DIP flexion; emphasize isolated DIP blocking with thumb stabilization, place-and-hold, and putty scraping/pinching for short bouts (a few minutes). Patient to inform clinic regarding preferred visit schedule.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		
JM to left IF PIP joint for extension		
PROM finger		
Intrinsic stretching		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units
		3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on January 14, 2026 at 10:29 am

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Progress Note

Visit No: 20
Date of Progress Note: 01/07/2026
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Patient reports finger is doing okay and elbows are okay overall. Has been taking medication the past three days due to increased back symptoms. Messaged rheumatology before Christmas; they require an in-person visit and earliest availability is March—patient plans to reach out again. Wears spring/central splint more in mornings/evenings; notes fingers can become numb if worn too long and can bend against the spring for relief. Continues to massage at home.

Patient Presentation

Patient/Caregiver Report: Patient states functional use is generally okay but continues to have difficulty opening items. Reports modifying workouts and activities; attends sculpt and hot power fusion classes with adjustments, avoids certain raises, and decreases lever to reduce strain. Worked one day and symptoms flared; notes poor flooring at workplace contributes to discomfort.

Primary Concern: Managing finger function and elbow symptoms while avoiding exacerbation; recent increase in back discomfort.

History of Present Condition: Inflammatory arthropathy with involvement of finger and elbow; history of back pain with prior MRI showing pars defect; psoriatic disease noted with spine involvement. Rheumatology follow-up pending for medication discussion.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Reduce soreness and maintain/improve finger motion and function while continuing modified exercise.

Other Services Received: Rheumatology consultation pending; patient contacted clinic and awaits March appointment.

Additional Comments: Reports recent severe wind led to poor sleep for the household; plans to consider new work shoes due to hard hospital floors (currently alternates Brooks running shoes and Danskos; Danskos increase foot discomfort).

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Right middle finger region; right lateral elbow; left-sided back pain.

Pain Description: Finger generally okay; occasional sensation of extensor tendon crossing over MP joint without pain—clinician observed intermittent subluxation. Elbow symptoms recur with workload and certain movements. Back pain worsened recently; forward fold aggravates; improved while on Mobic previously, with increased back symptoms after stopping.

Alleviating Factors: Activity modification; decreased lever in workouts; massage; spring splint with occasional bending against resistance for relief; Mobic previously helped back symptoms.

Exacerbating Factors: Work days on hard flooring; forward fold positions; repetitive gripping; certain raises/upper extremity loading; prolonged splint wear causing numbness.

Interventions/Physical Therapy to Address Pain: Measured grip and pinch strength and swelling; advised against repetitive grip strengthening; encouraged targeted putty scraping for index finger glide; education on monitoring extensor tendon subluxation and potential need for relative motion orthosis and nighttime extension splinting if it worsens; continued activity modifications.

Additional Comments: Patient will inform rheumatology about tendon subluxation; surgical option exists but deferred unless necessary.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Additional Comments: Progress measurements taken: index finger ROM and extension remain about the same; clinician would like a bit more flexion. Grip strength showed slight decrease bilaterally compared side-to-side but remains consistent overall; pinch strength increased (noted examples: from 9 to 13 on one measure). Edema measurements: left/right comparisons recorded with mild puffiness despite visually minimal swelling (examples noted: 6.2 previously and currently at same sites; other sites measured at 6.0, 5.8 vs 6.2, 5.3 vs 5.7). Patient reports intermittent extensor digitorum communis tendon subluxation at the index MP with palpable crossover; clinician observed an event during session and discussed monitoring and potential relative motion orthosis with nighttime extension splinting if recurrent. Patient has been modifying workouts; tolerates shoulder/chest work, forward folds aggravate back. Spring splint use noted (kept in car; worn mornings/evenings); caution about numbness with prolonged wear and option to bend against spring briefly for relief.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Dominant Hand: Right

Pinch Test Instrument: Pinch gauge

Lateral Pinch	Meas. 1	Meas. 2	Units	Coefficient of Variance	Average
Left	16	13	lbs	10.34	14.50
Right	17		lbs	0.00	17.00

Comments: Lateral pinch increased compared to prior; noted improvement bilaterally.

Tip Pinch/ Pincer	Meas. 1	Units	Coefficient of Variance	Average
Left	15	lbs	0.00	15.00
Right	17	lbs	0.00	17.00

Patient Name: Katherine Holmes
 Date of Birth: 08/16/1982
 EMR ID: 50300063
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Comments: Tip pinch increased on both sides compared to prior.

Tripod Pinch	Meas. 1	Units	Coefficient of Variance	Average
Left	20	lbs	0.00	20.00
Right	18	lbs	0.00	18.00

Comments: Tripod pinch showed slight improvement bilaterally.

Grip Test Instrument: Jamar dynamometer

Power Grip	Meas. 1	Meas. 2	Meas. 3	Units	Coefficient of Variance	Average
Left	68	67	64	lbs	2.56	66.33
Right	70	70	70	lbs	0.00	70.00

Comments: Slight decrease in grip strength noted; right side remained consistent across trials.

Repeated Grip	Units
Left	lbs
Right	lbs
Rapid Exchange	Units
Left	lbs
Right	lbs

Five Level Grip	Meas. 3	Meas. 4	Meas. 5	Units	Coefficient of Variance	Average
Left				lbs		
Right	68	70	70	lbs	1.36	69.33

Comments: Grip strength showed slight decrease compared to prior but remains consistent side-to-side; clinician noted repetitive grip strengthening may not be ideal given inflammatory condition.

Additional Comments: Pinch measurements increased: noted values of 17 and increase from 9 to 13 on pinch tests.

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (3)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). No difficulty (1)

3: Carry a shopping bag or briefcase. Mild difficulty (2)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. No difficulty (1)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Mild difficulty (2)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Not at all (1)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem?
Not limited at all (1)

9: Arm, shoulder, or hand pain. Mild (2)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? No difficulty (1)

Disability/Symptom Score: 9

Additional Comments: Patient reports nighttime wind disturbance causing poor sleep; modifies exercise to reduce strain; uses finger splint intermittently due to numbness when worn too long.

Outcomes Disability/Symptom Score 9.091/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D3 (middle finger)

Metacarpophalangeal (MCP) Joint: 5.8 cm

Proximal Phalanx (P1): 5.3 cm

Proximal Interphalangeal (PIP) Joint: 5.7 cm

Intermediate Phalanx (P2): 5.4 cm

Comments: Measured today; visual swelling minimal but persistent puffiness noted.

2. **Location:** Right D2 (index finger)

Metacarpophalangeal (MCP) Joint: 6.2 cm

Proximal Phalanx (P1): 6 cm

Proximal Interphalangeal (PIP) Joint: 6 cm

Intermediate Phalanx (P2): 5.7 cm

Comments: Index finger measurements about the same as prior.

Additional Comments: Edema primarily at right middle finger today; side-to-side comparison shows right more swollen than left at corresponding points.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM D2
Flexion

Metacarpophalangeal (MCP) Joint 80 °

Proximal Interphalangeal (PIP) Joint 98 °

Distal Interphalangeal (DIP) Joint 70 °

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
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Metacarpophalangeal (MCP) Joint		90 °
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Proximal Interphalangeal (PIP) Joint	-6 °	80 °
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Distal Interphalangeal (DIP) Joint		63 °
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Additional Comments

Night extension splint adjusted for slightly increased extension; advised Coban under splint to improve fit. Emphasis on blocking exercises to improve tendon glide; flexion strap lower priority. Advised to avoid palm-down/wrist pronated exercises and avoid tight grasping of weights; focus on proximal strengthening. Patient reports stiffness in the morning without pain after night splinting.

FLWSHEET

Activities/Interventions

Additional Comments:

- Therapeutic Interventions:

- Objective reassessment/measurements of hand function: index finger ROM (extension/flexion) and edema; grip and pinch strength bilaterally; tendon observation for EDC subluxation at MP joint.
- Education on activity modification and home management: advised to avoid repetitive grip strengthening due to inflammatory condition; recommended use of putty specifically for scraping/pulling the index finger through to improve flexion; instruction on monitoring for tendon subluxation and to inform Rheumatology; discussed potential relative motion orthosis and nighttime extension splinting if subluxation persists; guidance on shoe/footwear considerations for symptom management at work; encouraged balanced sitting/standing and modifications during workouts.

- Parameters:

- Strength testing: grip trials bilaterally with side-to-side comparison; lateral and two-point pinch assessed (values noted to have increased; exact numbers not fully specified in dialogue beyond examples of 17 and increase from 9 to 13 on one measure).
- Edema measurements: circumferential measures at multiple points of fingers/hands (examples noted: 6.2, 6.0, 5.8, 5.3, 5.7 cm depending on side/landmark).
- ROM: index finger flexion/extension assessed; flexion targeted to improve.

- Patient Response:

- Tolerated assessments without pain; reports occasional sensation of tendon "crossing over" at MP without pain; functional

difficulty with opening items; overall strength described as adequate; swelling visually minimal but measurable puffiness present.

- Progression/Regression:

- Strength: noted increases in pinch strength (e.g., from 9 to 13; right and left up to ~17 on one measure); grip strength slightly decreased bilaterally today but consistent side-to-side.

- ROM: motion maintained; goal to achieve more index finger flexion.

- Plan adjustments: continue to avoid repetitive grip strengthening; emphasize targeted putty scraping for index finger flexion; monitor for EDC tendon subluxation—consider relative motion orthosis and nighttime extension splint if episodes increase; patient to notify Rheumatology of tendon symptoms; continue activity modifications during workouts and consider supportive footwear at work.

ASSESSMENT

Diagnosis

Summary: Progress note: Patient with inflammatory hand involvement (index finger) and lateral elbow symptoms reports finger and elbows "okay." Objective measures show motion essentially unchanged with desire for slightly more flexion; grip and pinch strengths increased bilaterally without loss of motion. Mild hand puffiness persists by circumferential measures despite minimal visual swelling. Patient is modifying activities and exercise classes; splints used more frequently, though spring splint may cause transient numbness; rheumatology follow-up pending (scheduled March). Patient remains appropriate for continued skilled therapy to optimize tendon glide, protect extensor mechanism (EDC at MP with intermittent subluxation sensation), and progress functional use while managing inflammation.

See code details above

Additional Comments: Precautions: intermittent EDC subluxation sensation at index MP; avoid repetitive grip strengthening; monitor splint-related numbness/circulation; activity modification during inflammatory phases. Continue emphasis on tendon gliding/blocking, functional modifications, and splinting strategies as needed; communicate findings to Rheumatology. Rehab potential: Good.

Problems & Goals

1. **Problem:** Decrease in function

Status: Active

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Four weeks

Progress: Strength measures improved bilaterally; motion stable; functional tolerance improving with modifications.

Status: Progressing

2. **Type:** Short term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Six weeks

Progress: Increasing grip/pinch strength; still some swelling; activity modifications in place.

Status: Progressing

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Six weeks

Progress: Motion unchanged with desire for more flexion; pain minimal; mild puffiness persists.

Status: Progressing

4. **Type:** Short term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

Progress: Functional use improving with exercise modification; minimal pain reported.

Status: Progressing

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Four weeks

Progress: Grip/pinch increased; swelling measurements stable to slightly improved; tolerance better.

Status: Progressing

6. **Type:** Short term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Six weeks

Progress: Elbows reported "okay"; patient modifies lifts/raises; continued monitoring advised.

Status: Progressing

Rehabilitation Potential: Good

Additional Comments: Rehab potential: Good. Patient demonstrating improved strength and stable motion, with ongoing mild swelling and need for activity/splint modifications. Goals overall progressing; continued focus on tendon glide, controlled strengthening, and protection of extensor mechanism is indicated.

Patient/Caregiver Education

Education Provided: Discussed monitoring for index MP EDC tendon subluxation; advised activity modification and avoidance of repetitive heavy gripping; continue blocking/tendon gliding and functional use while protecting joints. Reviewed splint use (night/relative motion/spring) with caution for numbness and option to gently bend against spring for relief; increase wear as tolerated without compromising circulation. Reinforce communication with Rheumatology regarding tendon behavior and inflammatory symptoms.

Additional Comments: Emphasized symptom monitoring, swelling checks, and gradual progression of activities; maintain modifications during classes and work; follow-up with Rheumatology in March or sooner if symptoms escalate.

PLAN

Planned Treatment and Schedule

Approach: Begin/continue plan as outlined

Planned Procedures: Patient Education

Additional Comments: Continue per plan of care; monitor for signs of extensor tendon subluxation at the MP joint and consider relative motion orthosis with nighttime extension splinting if episodes persist; maintain targeted index-finger tendon-glide/blocking exercises; avoid repetitive gripping; continue exercise modifications; patient to follow up with Rheumatology (appointment anticipated in March).

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		

JM to left IF PIP joint for extension

PROM finger

Intrinsic stretching

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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CPT® Code	Modifier	Units
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units
		3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on January 7,
2026 at 4:37 pm

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
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Occupational Therapy Daily Note

Visit No: 19
Date of Daily Note: 12/31/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports night finger splint tends to slip off, especially when using a sleeve; has not tried Coban yet. Wears splint overnight; finger feels stiff on waking but not painful. Open to minor adjustment to increase extension; clinician adjusted spring splint and reviewed flexion strap tension. Notes near-full passive flexion; focusing on blocking for tendon glide. Plans to continue yoga. Contacted rheumatology about elbow meds but was told an in-person visit is needed before starting a new medication. Using meloxicam (Mobic/Namovic) only intermittently; unsure about timing (morning vs bedtime). Feels nighttime glutathione/antioxidant supplement may help morning symptoms. Avoiding wrist-down/palm-down exercises and tight gripping; focusing on proximal work.

Patient Presentation

Patient/Caregiver Report: Continues home program; notes improved function compared to prior weeks. Finger has mild morning stiffness; elbow soreness persists. Attended workouts with modifications (no weights for biceps/skull crushers). Performing eccentric exercises, scap raises, supination/pronation with a 2 lb weight.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Reduce elbow soreness, maintain/improve finger mobility and extension.

Additional Comments: Adheres better to exercises when working out at home; finds it harder to complete after yoga sessions.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Right middle finger (PIP region); right lateral elbow.

Pain Description: Finger: morning stiffness without pain; ongoing active extension limitation. Elbow: lateral pain provoked with certain resisted positions/movements; "sings/talks" with use.

Alleviating Factors: Movement during the day/yoga; proximal-focused exercises; intermittent meloxicam; nighttime antioxidant supplement perceived to help morning symptoms; splinting with adjustments planned (Coban under splint).

Exacerbating Factors: Palm-down/wrist-dominant lifts, tight gripping, biceps/triceps work; general upper-extremity loading.

Interventions/Physical Therapy to Address Pain: Education to avoid palm-down lifts and tight gripping; emphasize proximal strengthening; continue blocking for tendon glide; use spring splint and night splint (with Coban) and deprioritize flexion strap; brief elbow treatment planned during session.

Additional Comments: Rheumatology follow-up pending before any new medication. Patient plans to trial Coban to improve splint adherence.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Summary: Patient notes night spring splint tends to slip off, especially with sleeve and morning dampness; has not tried Coban yet. After wearing splint overnight, finger feels stiff in morning but not painful. Therapist adjusted splint to increase extension slightly; advised adjusting flexion straps as motion improves. PIP extension deficit improved to about 4° (from 6° last visit). PIP flexion near 60°. Passive finger motion near full. Elbow pain present laterally with certain movements; patient intermittently using NSAID (Nabumetone) and taking it as needed rather than regularly.

Additional Comments: Coban recommended under splint to improve retention; prioritize spring splint and night splint use, with flexion strap lower priority. Focus on tendon gliding via blocking since passive flexion is near full, suggesting adhesions over joint restriction. Activity guidance reinforced: avoid palm-down positions and tight gripping; emphasize proximal strengthening; anticipate symptoms with biceps/triceps work. Brief elbow session performed; continue gentle soft-tissue work and tendon stretches as tolerated.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Lateral Pinch	Meas. 1		Coefficient of Variance		Average
Left	14		0.00		14.00
Right	15		0.00		15.00
Tip Pinch/ Pincer	Meas. 1		Coefficient of Variance		Average
Left	9		0.00		9.00
Right	15		0.00		15.00
Tripod Pinch	Meas. 1		Coefficient of Variance		Average
Left	20		0.00		20.00
Right	19		0.00		19.00
Power Grip	Meas. 1	Meas. 2	Meas. 3	Coefficient of Variance	Average
Left	76	72	66	5.76	71.33
Right	72	78	75	3.27	75.00

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (4)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). No difficulty (1)
3: Carry a shopping bag or briefcase. No difficulty (1)
4: Wash your back. No difficulty (1)
5: Use a knife to cut food. No difficulty (1)
6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Moderate difficulty (3)
7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Not at all (1)
8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem?
 Not limited at all (1)
9: Arm, shoulder, or hand pain. None (1)
10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)
11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? Mild difficulty (2)
Disability/Symptom Score: 9
Outcomes Disability/Symptom Score 9.091/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D3 (middle finger)
Proximal Phalanx (P1): 5.8 cm
Proximal Interphalangeal (PIP) Joint: 5.7 cm
Intermediate Phalanx (P2): 5.4 cm
Comments: Slightly more swelling noted today, approximately 1 mm increase across measures.
2. **Location:** Left D2 (index finger)
Proximal Phalanx (P1): 6.2 cm
Proximal Interphalangeal (PIP) Joint: 6 cm
Intermediate Phalanx (P2): 6.3 cm

Additional Comments: Edema primarily at right middle finger today.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2
	Flexion

Metacarpophalangeal (MCP) Joint	80 °
Proximal Interphalangeal (PIP) Joint	98 °
Distal Interphalangeal (DIP) Joint	70 °

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
-----------------	-----------------	---------------

Metacarpophalangeal (MCP) Joint		90 °
Proximal Interphalangeal (PIP) Joint	-6 °	80 °
Distal Interphalangeal (DIP) Joint		63 °

Special Tests

Description: Middle finger PIP extension contracture improving: ~-4° today (was -6° last visit). PIP flexion approximately 60°. Passive flexion near full. Elbow symptoms persist laterally with certain movements; patient reports variable use of Namovic and activity-related irritation.

Palpation

Description: Lateral elbow symptoms consistent with lateral epicondyle irritation (patient localizes pain more lateral/top of elbow).

Additional Comments

Night extension splint adjusted for slightly increased extension; advised Coban under splint to improve fit. Emphasis on blocking exercises to improve tendon glide; flexion strap lower priority. Advised to avoid palm-down/wrist pronated exercises and avoid tight grasping of weights; focus on proximal strengthening. Patient reports stiffness in the morning without pain after night splinting.

FLOWSHEET

Activities/Interventions

Additional Comments:

- Adjusted finger spring splint (PIP extension) to increase extension bias; instructed to wear overnight; add Coban wrap under sleeve for improved grip and reduced slippage
- Educated on flexion strap adjustments (unknot to modify tension); de-emphasize flexion strap use currently given near-full passive flexion; prioritize spring splint and night extension splint
- Performed/IP reviewed PIP joint measurements: extension improved from -6° to ~-4°; flexion ~60° at affected joint
- Instructed on tendon gliding/blocking for digit (PIP/DIP) to promote differential tendon excursion; emphasize blocking drills over passive flexion given near-full passive flexion
- Advised activity modification for lateral elbow pain: avoid palm-down (pronated) wrist-loaded exercises; avoid tight gripping of weights; favor proximal strengthening; caution with biceps/triceps loading due to elbow involvement
- Recommended trial of Coban under splint sleeve and continued soft tissue techniques to address soft tissue tightness
- Medication discussion: patient using nabumetone (Namovic) PRN; no change initiated today; advised to take it consistently only if directed by prescriber
- Home program updated: nightly spring splint, night extension splint use, daytime tendon blocking sets; avoid wrist-down strengthening; monitor symptoms during yoga and workouts

ASSESSMENT

Diagnosis

Summary: PIP extension lag improved to about 4° (from -6° last visit). Passive flexion near full (~60° at affected joint). Night splint generally tolerated; morning stiffness without pain. Ongoing lateral elbow symptoms with activity; patient intermittently uses NSAID (Mobic) and is modifying exercises to avoid palm-down positions and tight grasping.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Prioritize active blocking/tendon gliding over passive flexion strap work; continue night and spring splints with Coban sleeve to improve fit; adjust splint tension as motion improves; avoid palm-down lifts and tight gripping; consider consistent NSAID use per provider guidance if symptoms flare; focus on proximal strengthening while protecting elbows.

Summary

Summary of Findings: Improved PIP extension to ~4° lag with near-full passive flexion (~60°). Night splint fit adjusted; advised Coban for slippage. Elbow pain provoked by resisted/wrist-pronated activities; patient modifying form and activity. Findings consistent with tendon adhesion limiting active motion more than joint contracture.

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Clinical Impressions/Conclusions: Course indicates improving PIP extension with near-full passive ROM, consistent with adhesions restricting active motion more than joint stiffness. Elbow symptoms lateral and activity-provoked; modifications helping. Continued splint optimization and active-focused therapy expected to yield further gains.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to optimize splint fit, reduce edema/irritation, improve tendon excursion and PIP extension, manage elbow symptoms, and restore functional hand use.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Reassess splints next visit; continue active blocking/tendon glides; lower priority on passive flexion strap; maintain proximal strengthening with elbow-protective technique.

Problems & Goals

1. **Problem:** Decrease in function

Status: Active

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Four weeks

Progress: Patient reports finger feels better than last week; still some stiffness and extension lag about 10 degrees.

Status: Progressing

2. **Type:** Short term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Six weeks

Progress: Ongoing stiffness; improved tolerance to daily tasks; no daily need for NSAID.

Status: Progressing

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Six weeks

Progress: Near full passive fist with mild stiffness; symptoms improving.

Status: Progressing

4. **Type:** Short term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

Progress: Functional use improving; intermittent stiffness persists.

Status: Progressing

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Four weeks

Progress: Reports less stiffness and better tolerance with activities; still limited extension today (~10 degrees).

Status: Progressing

6. **Type:** Short term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Six weeks

Progress: Elbow less tender; maintaining upper body activity with modifications.

Status: Progressing

Rehabilitation Potential: Good

Additional Comments: Finger extension improving (from -6 to ~-4); near full passive flexion; morning stiffness without pain. Elbow symptoms persist laterally; advised to avoid palm-down lifts and tight gripping; emphasize proximal strengthening; continue activity with modifications. Using night and spring splints; adjusted for more extension; consider Coban under splint for retention.

Patient/Caregiver Education

Education Provided: Reinforced home program emphasizing spring and night splints; adjusted splint for increased extension; suggested Coban under splint for fit. Prioritize blocking exercises to improve tendon glide; lower priority on flexion strap. Advised to avoid palm-down lifts and tight grasping; focus on proximal strengthening; biceps/triceps may provoke symptoms. NSAID (Namovic) use discussed; patient taking intermittently.

Additional Comments: Reviewed symptom monitoring and activity modifications; recommended Coban for splint slippage; follow-up not scheduled today.

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: • Continue per plan of care; monitor swelling and ROM of the middle finger (PIP extension previously ~ -6°, improved to ~ -4° today; passive flexion near 60° and passive ROM near full).

- Emphasize frequent active blocking exercises throughout the day to improve tendon glide; deprioritize flexion strap/passive stretching since passive ROM is near full.
- Continue nighttime finger splint and LMB/spring splint; today adjusted toward slightly more extension for improved fit—use finger sleeve/Coban underneath to reduce slippage and protect skin; patient to trial Coban.
- For elbow: maintain proximal strengthening; avoid tight gripping and palm-down (pronated) lifting; patient may continue NSAID (meloxicam/"Namovic") trial as needed; consider one additional shockwave treatment to the elbow if symptoms warrant; reassess next visit.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		



CPT® Code	Modifier	Units
JM to left IF PIP joint for extension PROM finger Intrinsic stretching 97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		3 Direct Units 3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on December 31, 2025 at 8:46 am

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Daily Note

Visit No: 18
Date of Daily Note: 12/24/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports right middle finger feels better than last week with passive motion still near full; notes active extension plateau with PIP extension deficit measuring about 10° today (slightly less motion than prior visit when it was under 10°). Elbow “doing fine,” less tender overall; completed push-ups and a full-body workout with only end-of-workout awareness. Has largely avoided Mobic for ~2 months but took it twice since ~12/7; open to trying a short course for a few weeks to calm inflammation. Night finger splint feels less effective lately; developed a small sore on dorsal finger, sleeve under splint causes it to slip off—plans to bring splints in for reassessment/remolding. Contacted rheumatology recently (no response yet); discussed interest in anti-inflammatory strategies and mentioned hearing GLP-1s may help inflammation in psoriatic arthritis. Continues proximal strengthening; reduced upper-body lifting frequency to once weekly to limit flare-ups; plans yoga today. Follow-up scheduled next Wednesday.

Patient Presentation

Patient/Caregiver Report: Continues home program; notes improved function compared to prior weeks. Finger has mild morning stiffness; elbow soreness persists. Attended workouts with modifications (no weights for biceps/skull crushers). Performing eccentric exercises, scap raises, supination/pronation with a 2 lb weight.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Reduce elbow soreness, maintain/improve finger mobility and extension.

Additional Comments: Adheres better to exercises when working out at home; finds it harder to complete after yoga sessions.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Right middle finger PIP region; lateral elbow.

Pain Description: Finger stiffness with active extension limitation; elbow symptoms improved with activity—only mild awareness at end of workout; less tenderness overall.

Alleviating Factors: Activity/proximal strengthening, yoga; intermittent Mobic (used twice since ~12/7); night splinting historically helpful though currently less effective.

Exacerbating Factors: Heavier/upper-body workouts (e.g., end of sessions); possible cold weather; palm-down lifting/rows—advised to avoid.

Interventions/Physical Therapy to Address Pain: Continuing blocking and proximal strengthening; plan to adjust/remold night splint; brief elbow treatment during session; activity modification (avoid upright rows/palm-down lifts).

Additional Comments: Reached out to rheumatology; considering medical options beyond methotrexate history; interested in anti-inflammatory effects of GLP-1s; will bring splints next visit for evaluation.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Summary: Patient reports finger feels better than last week; elbow doing okay. PIP extension deficit about 10° today (similar to last visit). Passive finger motion near full with mild stiffness. Night extension splint not working as well; causes a sore dorsally and tends to fall off when worn with sleeve due to morning swelling.

Additional Comments: Passive finger motion remains near full, indicating soft tissue/tendon adhesions more than joint restriction. Elbow less tender; patient tolerating push-ups and upper-body work with some caution. Advised to continue proximal strengthening and avoid palm-down lifts/upright rows. Plans to bring night and spring splints next visit for reassessment/remolding.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Lateral Pinch	Meas. 1		Coefficient of Variance		Average
Left	14		0.00		14.00
Right	15		0.00		15.00
Tip Pinch/ Pincer	Meas. 1		Coefficient of Variance		Average
Left	9		0.00		9.00
Right	15		0.00		15.00
Tripod Pinch	Meas. 1		Coefficient of Variance		Average
Left	20		0.00		20.00
Right	19		0.00		19.00
Power Grip	Meas. 1	Meas. 2	Meas. 3	Coefficient of Variance	Average
Left	76	72	66	5.76	71.33
Right	72	78	75	3.27	75.00

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (4)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). No difficulty (1)
3: Carry a shopping bag or briefcase. No difficulty (1)
4: Wash your back. No difficulty (1)
5: Use a knife to cut food. No difficulty (1)
6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Moderate difficulty (3)
7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Not at all (1)
8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem?
Not limited at all (1)
9: Arm, shoulder, or hand pain. None (1)
10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)
11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? Mild difficulty (2)
Disability/Symptom Score: 9
Outcomes Disability/Symptom Score 9.091/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

- 1. Location:** Right D3 (middle finger)
Proximal Phalanx (P1): 5.8 cm
Proximal Interphalangeal (PIP) Joint: 5.7 cm
Intermediate Phalanx (P2): 5.4 cm
Comments: Slightly more swelling noted today, approximately 1 mm increase across measures.
- 2. Location:** Left D2 (index finger)
Proximal Phalanx (P1): 6.2 cm
Proximal Interphalangeal (PIP) Joint: 6 cm
Intermediate Phalanx (P2): 6.3 cm

Additional Comments: Edema primarily at right middle finger today.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Flexion
------------------	---------------

Metacarpophalangeal (MCP) Joint	80 °
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Proximal Interphalangeal (PIP) Joint	98 °
--------------------------------------	------

Distal Interphalangeal (DIP) Joint	70 °
------------------------------------	------

AROM of Left Hand

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Left Digit AROM	D2 Extension	D2 Flexion
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Metacarpophalangeal (MCP) Joint		90 °
Proximal Interphalangeal (PIP) Joint	-6 °	80 °
Distal Interphalangeal (DIP) Joint		63 °

Special Tests

Description: PIP extension contracture (middle finger): about -10° today (was under 10° last time). Passive motion of the finger: near full with mild stiffness. Elbow: less tender per patient report.

Palpation

Description: Tender over the lateral epicondyle left elbow

Additional Comments

Mild pain with resisted MF extension test
No pain with resisted wrist extension

FLWSHEET

Activities/Interventions

Additional Comments:

- Assess passive and active ROM of right finger (post-op) with joint mobilization; note extension ~10° lag; gentle passive flexion/extension for tissue tolerance
- Provide manual soft-tissue mobilization to right elbow region for lateral elbow pain; brief session (~5 minutes) focusing on proximal forearm extensors
- Educate on activity modification: avoid upright rows and palm-down (pronated) lifts to reduce lateral elbow irritation
- Cue performance of proximal strengthening: incline/modified push-ups for shoulder/scapular stability; 2–3 sets to tolerance
- Reinforce HEP: daily gentle finger ROM (composite flexion/extension) 10–15 reps, 2–3 sets; elbow/wrist extensor stretching 30–45 sec holds x 2–3
- Advise trial of short course of anti-inflammatory (per patient's current Mobic use) for a few weeks to calm irritation; defer medication management to rheumatology
- Instruct to bring night and spring finger splints next session for assessment and potential remolding to accommodate increased motion
- Review shockwave history to lateral elbow (last session ~10 days ago); consider additional treatment pending response
- Encourage graded return to upper-body workouts 1x/week initially; emphasize full-body sessions holding a single weight vs bilateral loading as tolerated
- Promote general activity: yoga session today for mobility and proximal endurance; self-monitor symptoms and modify as needed

ASSESSMENT

Diagnosis

Summary: PIP extension lag roughly 10° today (slightly worse than <10° last visit). Passive finger motion remains near full with mild passive stiffness, supporting tendon adhesion/scarring over joint restriction. Patient reports reduced need for daily Mobic and overall improved elbow symptoms; continues exercises and uses splints though night splint feels less effective and causes minor dorsal skin irritation.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Emphasize active blocking over passive stretching; continue night splinting but bring splints for assessment/remolding due to comfort/fit issues; maintain proximal strengthening while avoiding palm-down lifts/upright rows; consider short trial of daily NSAID to calm inflammation as discussed.

Summary

Summary of Findings: Slight regression in PIP extension (about 10° lag) with near-full passive ROM and mild passive stiffness; elbow discomfort overall improved. Night splint less effective/causing minor irritation. Findings remain consistent with tendon adhesion limiting active motion more than joint contracture.

Therapeutic Outcome: Other

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Clinical Impressions/Conclusions: Presentation continues to align with tendon adhesion/scarring limiting active extension despite near-full passive ROM. Elbow symptoms trending better. High adherence to HEP and splinting; expect continued gains with guided progression and splint optimization.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to optimize splint fit, reduce edema/irritation, improve tendon excursion and PIP extension, manage elbow symptoms, and restore functional hand use.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Will reassess splints next session; continue active-focused tendon gliding/blocking and proximal strengthening with activity modification.

Problems & Goals

1. **Problem:** Decrease in function

Status: Active

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Four weeks

Progress: Patient reports finger feels better than last week; still some stiffness and extension lag about 10 degrees.

Status: Progressing

2. **Type:** Short term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

Progress: Ongoing stiffness; improved tolerance to daily tasks; no daily need for NSAID.

Status: Progressing

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Four weeks

Progress: Near full passive fist with mild stiffness; symptoms improving.

Status: Progressing

4. **Type:** Short term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

Progress: Functional use improving; intermittent stiffness persists.

Status: Progressing

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Four weeks

Progress: Reports less stiffness and better tolerance with activities; still limited extension today (~10 degrees).

Status: Progressing

6. **Type:** Short term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Progress: Elbow less tender; maintaining upper body activity with modifications.

Status: Progressing

Rehabilitation Potential: Good

Additional Comments: Finger feels better than last week; passive motion near full with mild stiffness; active extension lag about 10 degrees. Elbow symptoms improving; advised to avoid palm-down lifts/upright rows and continue proximal strengthening. Considering trial of NSAID (Mobic) for a few weeks to reduce inflammation.

Patient/Caregiver Education

Education Provided: Reinforced home program and proximal strengthening; advised to avoid upright rows and palm-down lifting; continue push-ups as tolerated. Discussed trial of NSAID for a few weeks to calm inflammation. Instructed to bring night and spring splints for assessment/remolding.

Additional Comments: Reviewed symptom monitoring and activity modifications; follow-up scheduled next Wednesday.

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: • Continue per plan of care; monitor swelling and ROM of the middle finger (PIP extension currently ~ -10° today; passive ROM remains near full).

- Emphasize frequent active blocking exercises throughout the day; deprioritize passive stretching since passive ROM is near full.
- Continue nighttime finger splint; patient to bring night splint and LMB spring splint next visit for assessment/remolding to improve fit and protect skin (use finger sleeve/Coban as tolerated; monitor for sores).
- For elbow: maintain proximal strengthening and pushing movements (e.g., push-ups); avoid upright rows and palm-down (pronated) lifting; consider a short trial of daily NSAID (e.g., mobic) for a few weeks to calm inflammation; reassess next visit.
- Consider one additional shockwave treatment to the elbow if symptoms warrant; reassess timing based on response since prior session ~10 days ago.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		

JM to left IF PIP joint for extension

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



CPT® Code	Modifier	Units
PROM finger		
Intrinsic stretching		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units
		3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on December 24, 2025 at 11:22 am

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Daily Note

Visit No: 17
Date of Daily Note: 12/17/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports week marked by increased finger swelling and feeling more restricted in fully straightening the middle finger; notes splint leaves marks and moisture at night—agrees to try finger sleeves under splint. States night splint helpful overall. Measurements today show slight increase in PIP flexion contracture (from -6 to -8°) with improved DIP flexion (from 60 to 64°). Elbow symptoms overall better with exercise; had brief intermittent sharp shooting pains lasting ~5 seconds without clear provocation. Acknowledges diet lapses over the past week and suspects weather/holiday sweets may be contributing to swelling. Considering contacting rheumatology after the New Year to discuss medication options; hesitant to start anything during holidays. Using Mobic intermittently for swelling but deferred around recent shockwave treatment from chiropractor; expressed frustration with unexpected treatment charges. Adherent to blocking exercises; passive ROM near full; continues to question long-term prognosis for full straightening.

Patient Presentation

Patient/Caregiver Report: Continues home program; notes improved function compared to prior weeks. Finger has mild morning stiffness; elbow soreness persists. Attended workouts with modifications (no weights for biceps/skull crushers). Performing eccentric exercises, scap raises, supination/pronation with a 2 lb weight.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Reduce elbow soreness, maintain/improve finger mobility and extension.

Additional Comments: Adheres better to exercises when working out at home; finds it harder to complete after yoga sessions.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Right middle finger (PIP) stiffness/swelling; lateral elbow region; forearm discomfort.

Pain Description: Intermittent sharp, brief shooting pain from lateral elbow region proximally (~5 seconds); general soreness previously, with current improvement overall.

Alleviating Factors: Night splinting for finger; activity as tolerated; intermittent Mobic (held within 24 hrs of shockwave).

Exacerbating Factors: Diet lapses/sweets possibly increasing swelling; weather/barometric changes; occasional unprovoked episodes.

Interventions/Physical Therapy to Address Pain: Continues blocking exercises; advised to use finger sleeves under night splint and maintain splint use; brief elbow treatment planned today.

Additional Comments: Patient plans to message rheumatology to discuss medication options after New Year; hesitant about starting meds during holidays and expresses concerns based on family experience with methotrexate.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Summary: Patient reports increased finger swelling this week; night extension splint feels tighter and leaves marks—provided finger sleeves to wear under splint. Clinician notes slight increase in edema at the middle finger with small regression in PIP extension but improved DIP flexion compared to last visit. Passive finger motion remains near full, suggesting tendon scarring rather than joint restriction.

Additional Comments: Patient demonstrates almost full passive finger motion. Scar tissue binding is noted in the finger, particularly affecting the flexor tendon.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Lateral Pinch	Meas. 1		Coefficient of Variance		Average
Left	14		0.00		14.00
Right	15		0.00		15.00
Tip Pinch/ Pincer	Meas. 1		Coefficient of Variance		Average
Left	9		0.00		9.00
Right	15		0.00		15.00
Tripod Pinch	Meas. 1		Coefficient of Variance		Average
Left	20		0.00		20.00
Right	19		0.00		19.00
Power Grip	Meas. 1	Meas. 2	Meas. 3	Coefficient of Variance	Average
Left	76	72	66	5.76	71.33
Right	72	78	75	3.27	75.00

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (4)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). No difficulty (1)

3: Carry a shopping bag or briefcase. No difficulty (1)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. No difficulty (1)

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Moderate difficulty (3)
7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Not at all (1)
8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem? Not limited at all (1)
9: Arm, shoulder, or hand pain. None (1)
10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)
11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? Mild difficulty (2)
Disability/Symptom Score: 9
Outcomes Disability/Symptom Score 9.091/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

- 1. Location:** Right D3 (middle finger)
Proximal Phalanx (P1): 5.8 cm
Proximal Interphalangeal (PIP) Joint: 5.7 cm
Intermediate Phalanx (P2): 5.4 cm
Comments: Slightly more swelling noted today, approximately 1 mm increase across measures.
- 2. Location:** Left D2 (index finger)
Proximal Phalanx (P1): 6.2 cm
Proximal Interphalangeal (PIP) Joint: 6 cm
Intermediate Phalanx (P2): 6.3 cm

Additional Comments: Edema primarily at right middle finger today.

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Flexion
------------------	---------------

Metacarpophalangeal (MCP) Joint	80 °
---------------------------------	------

Proximal Interphalangeal (PIP) Joint	98 °
--------------------------------------	------

Distal Interphalangeal (DIP) Joint	70 °
------------------------------------	------

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
-----------------	-----------------	---------------

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
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Metacarpophalangeal (MCP) Joint	90 °
Proximal Interphalangeal (PIP) Joint	-6 ° 80 °
Distal Interphalangeal (DIP) Joint	63 °

Special Tests

Description: PIP extension contracture measurement (middle finger): -8° today (was -6° last visit). DIP flexion (middle finger): ~64° today (was ~60° last visit).

Palpation

Description: Tender over the lateral epicondyle left elbow

Additional Comments

Mild pain with resisted MF extension test
No pain with resisted wrist extension

FLWSHEET

Activities/Interventions

Additional Comments:

- Assess finger ROM: DIP extension approx -8 degrees; DIP flexion improved to 64 degrees, passive motion checked
- Perform eccentric scap raises: bilateral shoulders/scapulae, eccentric focus, patient-reported home program
- Perform forearm pronation/supination: with 2 lb dumbbell, controlled tempo, eccentric emphasis
- Instruct elbow-focused eccentric strengthening: triceps (skull crusher pattern) without external load per tolerance
- Educate on activity modification during group workouts: perform biceps and triceps exercises without added weight to reduce elbow soreness
- Review HEP adherence strategies: prioritize home routine on non-yoga days to ensure completion
- Monitor symptom response: elbow soreness > hand; morning finger stiffness reported

ASSESSMENT

Diagnosis

Summary: Patient presents with mild increase in finger swelling and a slight regression in PIP extension (now approximately -8° from -6° last visit), while DIP flexion has improved (from ~60° to ~64°). Therapist notes near-full passive motion suggesting tendon scarring rather than joint restriction, with night splinting helpful; elbow symptoms are overall better with occasional brief shooting pains. Patient is engaging in prescribed blocking exercises and reports functional adherence to splint use.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Continue focus on active blocking over passive stretching; maintain night splinting and monitor swelling.

Summary

Summary of Findings: Slight increase in PIP extension lag with modest DIP flexion gains and mild swelling; elbow discomfort is improved overall with rare, brief sharp pains. Passive ROM remains nearly full, supporting tendon scarring as primary limiter

rather than joint contracture.

Therapeutic Outcome: Other

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Clinical Impressions/Conclusions: Findings are consistent with tendon adhesion/scarring limiting active extension despite near-full passive motion; small ROM fluctuations correlate with swelling. Patient demonstrates good adherence and benefits from splinting and targeted exercises.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to reduce edema, improve tendon excursion and PIP extension, address intermittent elbow pain, and restore functional hand use.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Patient remains a good candidate for ongoing skilled hand therapy given objective changes, good compliance, and potential for further ROM and functional gains with guided progression.

Problems & Goals

1. **Problem:** Decrease in function

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Two weeks

4. **Type:** Long term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Two weeks

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: • Continue per plan of care; monitor swelling and ROM of the middle finger (PIP extension currently ~-8°; DIP flexion ~64°).

• Emphasize active blocking exercises frequently throughout the day; deprioritize passive stretching since passive ROM is near full.

• Continue wearing the nighttime finger splint; use finger sleeves/"finger socks" or Coban underneath to reduce moisture/pressure; monitor skin and adjust fit as needed.

- Brief elbow treatment performed today; continue to monitor intermittent sharp elbow pain with activities; patient able to deadlift with neutral grip. Patient to bring LMB spring splint next visit to assess fit.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		
JM to left IF PIP joint for extension		
PROM finger		
Intrinsic stretching		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units
		3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on December 17, 2025 at 8:51 am

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Daily Note

Visit No: 16
Date of Daily Note: 12/10/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Reports overall improvement since last week; hand is okay with mild morning finger stiffness. Elbow remains the primary source of soreness.

Patient Presentation

Patient/Caregiver Report: Continues home program; notes improved function compared to prior weeks. Finger has mild morning stiffness; elbow soreness persists. Attended workouts with modifications (no weights for biceps/skull crushers). Performing eccentric exercises, scap raises, supination/pronation with a 2 lb weight.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Reduce elbow soreness, maintain/improve finger mobility and extension.

Additional Comments: Adheres better to exercises when working out at home; finds it harder to complete after yoga sessions.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Elbow > hand/finger (morning stiffness in finger).

Pain Description: Soreness at the elbow; finger stiffness in the morning.

Alleviating Factors: Activity modification (avoiding weights for biceps/skull crushers).

Exacerbating Factors: Weighted bicep exercises/skull crushers; general upper extremity loading.

Interventions/Physical Therapy to Address Pain: Eccentric exercises, scap raises, and forearm supination/pronation with 2 lb weight as part of HEP.

Additional Comments: Patient balancing gym classes and home program; better adherence when exercising at home.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Additional Comments: Patient demonstrates almost full passive finger motion. Scar tissue binding is noted in the finger, particularly affecting the flexor tendon.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Lateral Pinch	Meas. 1		Coefficient of Variance		Average
Left	14		0.00		14.00
Right	15		0.00		15.00
Tip Pinch/ Pincer	Meas. 1		Coefficient of Variance		Average
Left	9		0.00		9.00
Right	15		0.00		15.00
Tripod Pinch	Meas. 1		Coefficient of Variance		Average
Left	20		0.00		20.00
Right	19		0.00		19.00
Power Grip	Meas. 1	Meas. 2	Meas. 3	Coefficient of Variance	Average
Left	76	72	66	5.76	71.33
Right	72	78	75	3.27	75.00

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (4)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). No difficulty (1)

3: Carry a shopping bag or briefcase. No difficulty (1)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. No difficulty (1)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Moderate difficulty (3)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Not at all (1)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem? Not limited at all (1)

9: Arm, shoulder, or hand pain. None (1)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? Mild difficulty (2)

Disability/Symptom Score: 9

Outcomes Disability/Symptom Score 9.091/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D2 (index finger)
Proximal Phalanx (P1): 5.8 cm
Proximal Interphalangeal (PIP) Joint: 5.7 cm
Intermediate Phalanx (P2): 5.4 cm
2. **Location:** Left D2 (index finger)
Proximal Phalanx (P1): 6.2 cm
Proximal Interphalangeal (PIP) Joint: 6 cm
Intermediate Phalanx (P2): 4.3 cm

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Flexion
------------------	------------

Metacarpophalangeal (MCP) Joint	80 °
Proximal Interphalangeal (PIP) Joint	98 °
Distal Interphalangeal (DIP) Joint	70 °

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
-----------------	--------------	------------

Metacarpophalangeal (MCP) Joint		90 °
Proximal Interphalangeal (PIP) Joint	-6 °	80 °
Distal Interphalangeal (DIP) Joint		60 °

Palpation

Description: Tender over the lateral epicondyle left elbow

Additional Comments

Mild pain with resisted MF extension test
No pain with resisted wrist extension

FLOWSHEET

Activities/Interventions

Additional Comments:

- Assess finger ROM: DIP extension approx -8 degrees; DIP flexion improved to 64 degrees, passive motion checked
- Perform eccentric scap raises: bilateral shoulders/scapulae, eccentric focus, patient-reported home program
- Perform forearm pronation/supination: with 2 lb dumbbell, controlled tempo, eccentric emphasis
- Instruct elbow-focused eccentric strengthening: triceps (skull crusher pattern) without external load per tolerance
- Educate on activity modification during group workouts: perform biceps and triceps exercises without added weight to reduce elbow soreness
- Review HEP adherence strategies: prioritize home routine on non-yoga days to ensure completion
- Monitor symptom response: elbow soreness > hand; morning finger stiffness reported

ASSESSMENT

Diagnosis

Summary: Patient reports overall improvement since prior sessions with residual soreness primarily at the left lateral elbow and mild morning stiffness in the involved finger. Objective measures show maintained PIP extension at approximately -8° and improved DIP flexion from 60° to 64°, indicating continued gains in digital motion. Therapist notes progress is consistent, with elbow symptoms persisting but manageable alongside ongoing exercise adherence.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: Patient continues prescribed eccentric exercises and light strengthening; modifies gym activities to avoid exacerbation.

Summary

Summary of Findings: Improving finger ROM with stable PIP extension (-8°) and increased DIP flexion (64°); elbow soreness persists but patient tolerates activity with modifications.

Response to Exercise/Training: Patient is responding positively to the prescribed exercises and therapy.

Clinical Impressions/Conclusions: Patient is progressing with improved finger motion and stable elbow symptoms; continues to demonstrate good adherence to exercises and modifications.

Therapy Cap Exception Justification: Continued skilled therapy is medically necessary to further improve tendon mobility and ROM, address persistent elbow pain, and restore higher-level functional use.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: Recommend continuation of therapy to build on ROM gains and progressively load elbow as tolerated.

Problems & Goals

1. **Problem:** Decrease in function

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

2. **Type:** Long term

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Two weeks

4. **Type:** Long term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Two weeks

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined

Additional Comments: Continue eccentric scap raises, supination/pronation with 2 lb weight; monitor elbow soreness; maintain HEP consistency on non-yoga days; continue progressing ROM measurements for finger (focus on extension and DIP flexion).

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		
JM to left IF PIP joint for extension		
PROM finger		
Intrinsic stretching		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		
		3 Direct Units
		3 Total Units

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT

License #OT.0002252

Electronically signed by Megan Brunell, OTR, CHT on December
10, 2025 at 1:27 pm

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Progress Note

Visit No: 15
Date of Progress Note: 12/03/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Doing better today.

Patient Presentation

Patient/Caregiver Report: The patient reports wearing the splint and experiencing improvement in finger extension and passive motion. They also mentioned scar tissue binding as a concern.

Date of Injury/Onset: 08/19/2023

Patient/Caregiver Goals: Improve finger mobility and reduce scar tissue binding.

Additional Comments: The patient discussed their experiences with shockwave therapy and their follow-up plans with the provider in December.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Finger and elbow.

Pain Description: Soreness and discomfort, particularly when performing certain exercises.

Alleviating Factors: Eccentric exercises and wearing the splint.

Exacerbating Factors: Bicep curls and holding weights.

Interventions/Physical Therapy to Address Pain: Instrument assisted soft tissue massage, joint mobilization, and vibration therapy.

Additional Comments: The patient is focusing on exercises that do not exacerbate symptoms and is following recommendations for scar tissue management.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Additional Comments: Patient demonstrates almost full passive finger motion. Scar tissue binding is noted in the finger, particularly affecting the flexor tendon.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Lateral Pinch	Meas. 1		Coefficient of Variance		Average
Left	14		0.00		14.00
Right	15		0.00		15.00
Tip Pinch/ Pincer	Meas. 1		Coefficient of Variance		Average
Left	9		0.00		9.00
Right	15		0.00		15.00
Tripod Pinch	Meas. 1		Coefficient of Variance		Average
Left	20		0.00		20.00
Right	19		0.00		19.00
Power Grip	Meas. 1	Meas. 2	Meas. 3	Coefficient of Variance	Average
Left	76	72	66	5.76	71.33
Right	72	78	75	3.27	75.00

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (4)

1: Open a tight or new jar. Mild difficulty (2)

2: Do heavy household chores (e.g., wash walls, floors). No difficulty (1)

3: Carry a shopping bag or briefcase. No difficulty (1)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. No difficulty (1)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Moderate difficulty (3)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Not at all (1)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem? Not limited at all (1)

9: Arm, shoulder, or hand pain. None (1)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? Mild difficulty (2)

Disability/Symptom Score: 9

Outcomes Disability/Symptom Score 9.091/100

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D2 (index finger)
Proximal Phalanx (P1): 5.8 cm
Proximal Interphalangeal (PIP) Joint: 5.7 cm
Intermediate Phalanx (P2): 5.4 cm
2. **Location:** Left D2 (index finger)
Proximal Phalanx (P1): 6.2 cm
Proximal Interphalangeal (PIP) Joint: 6 cm
Intermediate Phalanx (P2): 4.3 cm

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Flexion
------------------	---------------

Metacarpophalangeal (MCP) Joint	80 °
---------------------------------	------

Proximal Interphalangeal (PIP) Joint	98 °
--------------------------------------	------

Distal Interphalangeal (DIP) Joint	70 °
------------------------------------	------

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
-----------------	-----------------	---------------

Metacarpophalangeal (MCP) Joint		90 °
---------------------------------	--	------

Proximal Interphalangeal (PIP) Joint	-6 °	80 °
--------------------------------------	------	------

Distal Interphalangeal (DIP) Joint		60 °
------------------------------------	--	------

Additional Comments

Mild pain with resisted MF extension test
No pain with resisted wrist extension

ASSESSMENT

Diagnosis

Summary: The patient demonstrates improved range of motion in the index finger, with PIP extension at -12 degrees and DIP flexion at 60 degrees. The therapist notes that the scar tissue is likely binding the tendon, limiting motion. Left lateral elbow pain has improved but persists. She has been able to get back into some activities, but is still limited with heavy recreational activities.

See code details above

Additional Comments: The patient is progressing well with therapy and demonstrates compliance with the prescribed exercises and splint usage.

Problems & Goals

1. **Problem:** Decrease in function

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Two weeks

4. **Type:** Long term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Two weeks

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Additional Comments: Progress scar mgmt, ROM, and functional use.

CHARGE SUMMARY

CPT® Code

97140 - Manual Therapy

Modifier

GO

Units

1

Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.

JM to left IF PIP joint for extension



CPT® Code	Modifier	Units
PROM finger Intrinsic stretching 97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		3 Direct Units
		3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on December 3, 2025 at 4:28 pm

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Daily Note

Visit No: 13
Date of Daily Note: 11/19/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: Doing better today.

Patient Presentation

Patient/Caregiver Report: The patient reports wearing the splint and experiencing improvement in finger extension and passive motion. They also mentioned scar tissue binding as a concern.

Date of Injury/Onset: 08/19/2023

Date of Surgery: 08/20/2025

Patient/Caregiver Goals: Improve finger mobility and reduce scar tissue binding.

Additional Comments: The patient discussed their experiences with shockwave therapy and their follow-up plans with the provider in December.

Current Functional Limitations Impacting Prior Level of Function

General Tasks and Demands

Current Limitations: Carrying out daily routine

Comments: The patient mentioned difficulty with certain physical activities and exercises due to soreness and scar tissue.

Additional Comments: The patient is focusing on eccentric exercises and avoiding activities that exacerbate symptoms.

Pain Presentation

Pain Location(s): Finger and elbow.

Pain Description: Soreness and discomfort, particularly when performing certain exercises.

Alleviating Factors: Eccentric exercises and wearing the splint.

Exacerbating Factors: Bicep curls and holding weights.

Interventions/Physical Therapy to Address Pain: Instrument assisted soft tissue massage, joint mobilization, and vibration therapy.

Additional Comments: The patient is focusing on exercises that do not exacerbate symptoms and is following recommendations for scar tissue management.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Objective Findings

Summary: • PIP flexion measured at 75 degrees.

• DIP flexion measured at 60 degrees.

• PIP extension measured at negative 12 degrees, improved to negative 10 degrees after treatment.

Additional Comments: Patient demonstrates almost full passive finger motion. Scar tissue binding is noted in the finger, particularly affecting the flexor tendon.

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Lateral Pinch		Meas. 1		Coefficient of Variance	Average
Left		14		0.00	14.00
Right		19		0.00	19.00
Tip Pinch/ Pincer		Meas. 1		Coefficient of Variance	Average
Left		9		0.00	9.00
Right		15		0.00	15.00
Tripod Pinch		Meas. 1		Coefficient of Variance	Average
Left		18		0.00	18.00
Right		18		0.00	18.00
Power Grip	Meas. 1	Meas. 2	Meas. 3	Coefficient of Variance	Average
Left	52	45	44	7.57	47.00
Right	80	60	70	11.66	70.00

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (7)

1: Open a tight or new jar. Moderate difficulty (3)

2: Do heavy household chores (e.g., wash walls, floors). Mild difficulty (2)

3: Carry a shopping bag or briefcase. Mild difficulty (2)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. Mild difficulty (2)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Moderate difficulty (3)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Slightly (2)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem? Moderately limited (3)

9: Arm, shoulder, or hand pain. Moderate (3)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? Mild difficulty (2)

Disability/Symptom Score: 30

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Outcomes Disability/Symptom Score 29.546/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D2 (index finger)
Proximal Phalanx (P1): 5.8 cm
Proximal Interphalangeal (PIP) Joint: 5.7 cm
Intermediate Phalanx (P2): 5.4 cm
2. **Location:** Left D2 (index finger)
Proximal Phalanx (P1): 6.4 cm
Proximal Interphalangeal (PIP) Joint: 6.2 cm
Intermediate Phalanx (P2): 5.5 cm

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Flexion
------------------	---------------

Metacarpophalangeal (MCP) Joint	80 °
---------------------------------	------

Proximal Interphalangeal (PIP) Joint	98 °
--------------------------------------	------

Distal Interphalangeal (DIP) Joint	70 °
------------------------------------	------

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
-----------------	-----------------	---------------

Metacarpophalangeal (MCP) Joint		90 °
---------------------------------	--	------

Proximal Interphalangeal (PIP) Joint	-14 °	80 °
--------------------------------------	-------	------

Distal Interphalangeal (DIP) Joint		58 °
------------------------------------	--	------

Palpation

Description: Tender over the lateral epicondyle left elbow

Additional Comments

Mild pain with resisted MF extension test
No pain with resisted wrist extension

ASSESSMENT

Diagnosis

Summary: The patient demonstrates improved range of motion in the index finger, with PIP extension at -12 degrees and DIP flexion at 60 degrees. The therapist notes that the scar tissue is likely binding the tendon, limiting motion.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: The patient is progressing well with therapy and demonstrates compliance with the prescribed exercises and splint usage.

Summary

Summary of Findings: The patient has nearly full passive motion in the affected finger and is making progress in reducing the limitations caused by scar tissue.

Response to Exercise/Training: The patient is responding positively to the prescribed exercises and therapy.

Clinical Impressions/Conclusions: The patient is progressing well with therapy and demonstrates compliance with the prescribed exercises and splint usage.

Therapy Cap Exception Justification: The patient demonstrates medical necessity for continued therapy to address scar tissue and improve functional range of motion.

Rehabilitation Potential: Patient demonstrates good rehabilitation potential to attain established short-term and long-term goals, and to maximize functional independence.

Additional Comments: The patient is advised to continue therapy and follow up with the referring physician in December.

Problems & Goals

1. **Problem:** Decrease in function

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Two weeks

4. **Type:** Long term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Two weeks

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined
Additional Comments: Progress scar mgmt, ROM, and functional use.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		3 Direct Units
		3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on November 19, 2025 at 12:24 pm

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



Occupational Therapy Daily Note

Visit No: 12
Date of Daily Note: 11/12/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
Date of Injury/Onset: 08/19/2023
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: I am having more aching the past few days.

Patient Presentation

Date of Injury/Onset: 08/19/2023

Date of Surgery: 08/20/2025

Patient/Caregiver Goals: Decrease swelling and full ROM

Current Functional Limitations Impacting Prior Level of Function

Additional Comments: Pt is experiencing limitations including opening containers, helping kids get dressed, performing work duties.

Currently unable to do a plank with weight bearing onto the hand, unable to hold weight in the hand. Difficulty with work activities such as plugging in blankets and starting an IV.

Current Medications

Refer to eDocs

Occupation(s)

Work Classification

Additional Comments: Pt is a nurse. She enjoys snowboarding, yoga, and has three kids (14, 11, and 8 years old).

OBJECTIVE

Standardized Tests

Pinch/Grip Test

The Pinch & Grip set of tests are used to measure the strength of the hand and forearm muscles.

Lateral Pinch	Meas. 1	Coefficient of Variance	Average
Left	14	0.00	14.00
Right	19	0.00	19.00
Tip Pinch/ Pincer	Meas. 1	Coefficient of Variance	Average
Left	9	0.00	9.00
Right	15	0.00	15.00
Tripod Pinch	Meas. 1	Coefficient of Variance	Average

Patient Name: Katherine Holmes
 Date of Birth: 08/16/1982
 EMR ID: 50300063
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Left	18	0.00	18.00		
Right	18	0.00	18.00		
Power Grip	Meas. 1	Meas. 2	Meas. 3	Coefficient of Variance	Average
Left	52	45	44	7.57	47.00
Right	80	60	70	11.66	70.00

QuickDASH (Disabilities of the Arm, Shoulder, and Hand)

Description: The QuickDASH measures physical function and symptoms in people with any or multiple musculoskeletal disorders of the upper limb.

Please rate your pain level with activity. (7)

1: Open a tight or new jar. Moderate difficulty (3)

2: Do heavy household chores (e.g., wash walls, floors). Mild difficulty (2)

3: Carry a shopping bag or briefcase. Mild difficulty (2)

4: Wash your back. No difficulty (1)

5: Use a knife to cut food. Mild difficulty (2)

6: Recreational activities in which you take some force or impact through your arm, shoulder, or hand (e.g., golf, hammering, tennis). Moderate difficulty (3)

7: To what extent has your arm, shoulder, or hand problem interfered with your normal social activities with family, friends, neighbors, or groups? Slightly (2)

8: Were you limited in your work or other regular daily activities as a result of your arm, shoulder, or hand problem? Moderately limited (3)

9: Arm, shoulder, or hand pain. Moderate (3)

10: Tingling (pins and needles) in your arm, shoulder, or hand. None (1)

11: How much difficulty have you had sleeping because of the pain in your arm, shoulder, or hand? Mild difficulty (2)

Disability/Symptom Score: 30

Outcomes Disability/Symptom Score 29.546/100

Observation

Description: Volar incision healing well. Moderate edema.

Girth Measurements

Wrist/Hand/Digits

1. **Location:** Right D2 (index finger)

Proximal Phalanx (P1): 5.8 cm

Proximal Interphalangeal (PIP) Joint: 5.7 cm

Intermediate Phalanx (P2): 5.4 cm

2. **Location:** Left D2 (index finger)

Proximal Phalanx (P1): 6.4 cm

Proximal Interphalangeal (PIP) Joint: 6.2 cm

Intermediate Phalanx (P2): 5.5 cm

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM **D2 Flexion**

Metacarpophalangeal (MCP) Joint	80 °
Proximal Interphalangeal (PIP) Joint	98 °
Distal Interphalangeal (DIP) Joint	70 °

AROM of Left Hand

Left Digit AROM **D2 Extension** **D2 Flexion**

Metacarpophalangeal (MCP) Joint		90 °
Proximal Interphalangeal (PIP) Joint	-14 °	80 °
Distal Interphalangeal (DIP) Joint		58 °

Palpation

Description: Tender over the lateral epicondyle left elbow

Additional Comments

Mild pain with resisted MF extension test
 No pain with resisted wrist extension

ASSESSMENT

Diagnosis

Summary: Pt has some increased pain today. She has generalized aching. She has increased finger DIP flexion today. Her left lateral elbow pain persists.

See code details above

Summary

Therapy Cap Exception Justification: This section is to provide explanation of medical necessity of care for patients that are over their therapy cap

Problems & Goals

- Problem:** Decrease in function

Goals

- Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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Tel: (303) 953-3163 Fax: (303) 245-0726



2. **Type:** Long term
Description: Improve function to allow opening a new jar with adaptive equipment as needed
Target Timeframe: Four weeks
3. **Type:** Short term
Description: Improve function to allow making full fist without pain
Target Timeframe: Two weeks
4. **Type:** Long term
Description: Improve function to allow buttoning shirt without pain
Target Timeframe: Four weeks
5. **Type:** Short term
Description: Improve function to allow grasping light object with mild symptoms
Target Timeframe: Two weeks
6. **Type:** Long term
Description: Improve function to allow pouring a pitcher without pain
Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Additional Comments: Progress scar mgmt, ROM, and functional use.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: STM to hand		
Mini massager to left volar/dorsal IF PIP		
IASTYM - hand and left dorsal FA		
PROM IF flex/ext		
MWM left elbow for wrist extension/flexion		
JM to elbow RH		
97110 - Therapeutic Exercise	GO	2
Comment: HEP: Heat, edema management, tendon glides, IF prom		
US to volar finger .6 cont 3.3 mHz, 6 min, left lateral ep .8 cont 3.3 mHz, 6 min		
Finger splint to wear at night - fabricated and educated on proper wear and care - NP 5/14/25		
Initiated LMB orthosis for daytime intermittent use		
Educated on radial nerve glides, proximal stability strengthening, push ups		
		3 Direct Units
		3 Total Units

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT

License #OT.0002252

Electronically signed by Megan Brunell, OTR, CHT on November
12, 2025 at 1:13 pm

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

Mountain States Therapy - Wheat Ridge
4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
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Occupational Therapy Daily Note

Visit No: 12
Date of Daily Note: 11/05/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: It is feeling really sore today.

ASSESSMENT

Diagnosis

Summary: Pt has increased soreness today. She has increased pain with increased use. She is tolerating treatment well.

Patient demonstrates compliance with prescribed home exercise program (HEP): Yes

See code details above

Additional Comments: The patient is progressing well with therapy and demonstrates compliance with the prescribed exercises and splint usage.

Summary

Last Session: 11/05/2025

Therapy Cap Exception Justification: This section is to provide explanation of medical necessity of care for patients that are over their therapy cap

Problems & Goals

1. **Problem:** Decrease in function

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Two weeks

4. **Type:** Long term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Two weeks

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Plan: Begin plan as outlined
Additional Comments: Progress scar mgmt, ROM, and functional use.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: Performed soft tissue massage using instrument-assisted techniques on the volar hand and wrist extensors. Joint mobilization was applied to the radial humeral joint.		
97110 - Therapeutic Exercise	GO	2
Comment: Patient was instructed on eccentric strengthening exercises, blocking exercises, and gentle stretching for the affected areas.		
US to volar hand and left lateral elbow, 1.0 cont 3.3 mHz, 10 min		3 Direct Units
		3 Total Units

Megan Brunell, OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on November 26, 2025 at 5:49 pm

Occupational Therapy Daily Note

Visit No: 11

Date of Daily Note: 10/29/2025

Insurance Name: ATA/Holista

Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025

EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand

Treatment Diagnosis: M79.642: Pain in left hand

SUBJECTIVE

Patient Report

Summary: I am having more aching the past few days.

OBJECTIVE

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))
AROM of Right Hand

Right Digit AROM	D2 Flexion
Metacarpophalangeal (MCP) Joint	80 °
Proximal Interphalangeal (PIP) Joint	98 °
Distal Interphalangeal (DIP) Joint	70 °

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
Metacarpophalangeal (MCP) Joint		90 °
Proximal Interphalangeal (PIP) Joint	-14 °	80 °
Distal Interphalangeal (DIP) Joint		58 °

ASSESSMENT

Diagnosis

Summary: Pt has some increased pain today. She has generalized aching. She has increased finger DIP flexion today.
See code details above

Summary

Therapy Cap Exception Justification: This section is to provide explanation of medical necessity of care for patients that are over their therapy cap

Problems & Goals

1. **Problem:** Decrease in function

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Two weeks

4. **Type:** Long term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Two weeks

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Additional Comments: Progress scar mgmt, ROM, and functional use.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: STM to hand		
Mini massager to left volar/dorsal IF PIP		
IASTYM - hand and left dorsal FA		
PROM IF flex/ext		
MWM left elbow for wrist extension/flexion		
JM to elbow RH		
97110 - Therapeutic Exercise	GO	2
Comment: HEP: Heat, edema management, tendon glides, IF prom		

US to volar finger .6 cont 3.3 mHz, 6 min, left lateral ep .8 cont 3.3 mHz, 6 min



CPT® Code	Modifier	Units
Finger splint to wear at night - fabricated and educated on proper wear and care - NP 5/14/25 Initiated LMB orthosis for daytime intermittent use		
Educated on radial nerve glides, proximal stability strengthening, push ups		3 Direct Units 3 Total Units

Megan Brunell OTR, CHT

Megan Brunell, OTR, CHT
License #OT.0002252
Electronically signed by Megan Brunell, OTR, CHT on October
29, 2025 at 9:56 am

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
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Occupational Therapy Daily Note

Visit No: 10
Date of Daily Note: 10/22/2025
Insurance Name: ATA/Holista
Place of Service: 11 — Office

Date of Original Evaluation: 09/04/2025
EMR ID: 50300063

Medical Diagnosis: M79.642: Pain in left hand
Treatment Diagnosis: M79.642: Pain in left hand

OBJECTIVE

Range of Motion | Hands

Measures (Note - Deficits in Extension must be recorded as a negative value (e.g. -10))

AROM of Right Hand

Right Digit AROM	D2 Flexion
------------------	---------------

Metacarpophalangeal (MCP) Joint	80 °
---------------------------------	------

Proximal Interphalangeal (PIP) Joint	98 °
--------------------------------------	------

Distal Interphalangeal (DIP) Joint	70 °
------------------------------------	------

AROM of Left Hand

Left Digit AROM	D2 Extension	D2 Flexion
-----------------	-----------------	---------------

Metacarpophalangeal (MCP) Joint		90 °
---------------------------------	--	------

Proximal Interphalangeal (PIP) Joint	-14 °	80 °
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Distal Interphalangeal (DIP) Joint		45 °
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ASSESSMENT

Diagnosis

Summary: Pain continues to decrease. Pt has continued improvement in left lateral ep symptoms. She continues to have some IF ROM limitations, however PIP extension has improved.

See code details above

Summary

Therapy Cap Exception Justification: This section is to provide explanation of medical necessity of care for patients that are over their therapy cap

Problems & Goals

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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4045 Wadsworth Blvd Ste 210
Wheat Ridge CO 80033-4624
Tel: (303) 953-3163 Fax: (303) 245-0726



1. **Problem:** Decrease in function

Goals

1. **Type:** Short term

Description: Improve function to allow opening a previously opened jar with minimal symptoms

Target Timeframe: Two weeks

2. **Type:** Long term

Description: Improve function to allow opening a new jar with adaptive equipment as needed

Target Timeframe: Four weeks

3. **Type:** Short term

Description: Improve function to allow making full fist without pain

Target Timeframe: Two weeks

4. **Type:** Long term

Description: Improve function to allow buttoning shirt without pain

Target Timeframe: Four weeks

5. **Type:** Short term

Description: Improve function to allow grasping light object with mild symptoms

Target Timeframe: Two weeks

6. **Type:** Long term

Description: Improve function to allow pouring a pitcher without pain

Target Timeframe: Four weeks

Rehabilitation Potential: Good

PLAN

Planned Treatment and Schedule

Additional Comments: Progress scar mgmt, ROM, and functional use.

CHARGE SUMMARY

CPT® Code	Modifier	Units
97140 - Manual Therapy	GO	1
Comment: STM to hand		
Mini massager to left volar/dorsal IF PIP		
IASTYM - hand and left dorsal FA		
PROM IF flex/ext		
MWM left elbow for wrist extension/flexion		
JM to elbow RH		
97110 - Therapeutic Exercise	GO	2
Comment: HEP: Heat, edema management, tendon glides, IF prom		

US to volar finger .6 cont 3.3 mHz, 6 min, left lateral ep .8 cont 3.3 mHz, 6 min

Finger splint to wear at night - fabricated and educated on proper wear and care - NP 5/14/25

Initiated LMB orthosis for daytime intermittent use

Educated on radial nerve glides, proximal stability strengthening, push ups

3 Direct Units

Patient Name: Katherine Holmes
Date of Birth: 08/16/1982
EMR ID: 50300063
Referring Physician/NPP: Christopher Fender MD

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Wheat Ridge CO 80033-4624
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3 Total Units

Megan Brunell OTR, CHT

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Electronically signed by Megan Brunell, OTR, CHT on October
22, 2025 at 9:59 am